

NIHR PHIRST local government expression of interest (Eol)

Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: Chelmsford City Council's Exercise Referral Scheme: Active Health

Lead Information

Name: Mr Phil Abram

Job Position: Active Health Programme Lead

Department: Leisure & Heritage

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Organisation: Chelmsford City Council

Contact Information

Lead officer for intervention:

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Job Title: Active Health Programme Lead

Department: Leisure & Heritage

Organisation: Chelmsford City Council

Lead officer for this Eol:

Contact details the same as lead officer for intervention.

Second point of contact for this Eol:

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Job Title: Leisure and Heritage Services Manager

Department: Leisure & Heritage

Organisation: Chelmsford City Council

Supporting Director of Public Health (DPH) or Chief Executive Officer (CEO):

Chief Executive Officer:

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Job Title: Chief Executive

Organisation: Chelmsford City Council

Expression of Interest

Lead organisation(s):

Chelmsford City Council

Description of the intervention:

The Active Health Programme is a 12-week exercise referral intervention delivered by Chelmsford City Council's leisure sites. The scheme utilises a wide range of physical activities to focus on the prevention, management & rehabilitation of a wide range of health conditions for Chelmsford residents. Referrals are from primary and community healthcare providers, who identify residents with health conditions that

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may benefit from prescribed condition specific physical activity programmes & increased physical activity.

From April 2023-March 2025 the scheme has received 1025 referrals. There is a year-on-year increase of over 100 referrals which demonstrates the scheme is gaining traction with local healthcare providers. The scheme is therefore evidencing the ever-growing need for an accessible exercise referral scheme for healthcare professionals. 94.36% of participants that complete our initial supervised process commit to a 12-week membership, covering the entirety of the intervention. Of these participants 58% complete the 12-week intervention, 78% of these graduates commit to a discounted monthly membership.

Analysis of graduates indicated improvements in physiological measurements, such as, reduced blood pressure & BMI. Improvements in quality-of-life indicators are also highlighted, such as, increased perception of health, fitness & diet. 84.3% of graduates achieved their specified aims set at the beginning of the intervention. Graduates also display a 50% reduction in unscheduled healthcare visits while undertaking the 12-week intervention compared to the prior 12-weeks when not part of the Exercise Referral Scheme (ERS). Also, the leading training provider for exercise referral qualifications, The Wright Foundation, has highlighted our scheme as a centre for good practice and signposted other ERS's to view or scheme.

The key aims we wish to evaluate are long-term adherence to increased physical activity for graduates of the ERS. Measuring graduates that are 6-months post completion of the ERS and are 1) Achieving NHS physical guidelines per week 2) If, achieving 1 what type/s of physical activity are they using.

Rationale for evaluation: As evidenced by improvements in physiological outcomes, quality of life and reductions in unscheduled healthcare visits from graduates of the ERS. Has the 12-week ERS resulted in long-term behavioural change to physical activity and led to further improvements & management of the graduated health condition/s.

Geographical location(s) of the intervention:

Chelmsford, Essex. Population approximately 181,500 (2021 census).

Current status and timescale(s) of the intervention:

Ongoing

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

The Active Health Scheme the scheme is currently funded by the city council as part of its leisure programme. But to expand this further requires a level of external funding. We would be able to contribute £5000 as a contribution to this evaluation and provide staff resource and delivery expertise to support any evaluation.

Intervention Start Date:

1st of April 2023

Intervention End Date:

The intervention is a core part of leisure programme and is funded for the foreseeable future. For evaluation purposes we suggest March 2027 (TBD).

Impacts:

Improving long-term health management, increased physical activity with positive behavioural change and positive mental wellbeing. Reduced unscheduled healthcare visits, easing pressure and costs on both primary and community healthcare providers. The scheme has captured many testimonials where lives have been transformed.

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Improve networking with health organisations due to increased credibility of scheme from evidence gathered on the effectiveness of ERS as a health intervention.

Public/community involvement or engagement:

Ongoing collaboration with Chelmsford's local healthcare service providers. Pilot project approaching completion with Chelmsford City Council's Public Health & Protection Team on the financial barriers for residents to join Chelmsford's ERS.

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

Mid and South Essex Integrated Care System and Livewell Chelmsford.

Weblinks:

<https://www.chelmsford.gov.uk/leisure-centres/what/gym/active-health/>

Additional information:

None

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Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: The Barnsley Older People Physical Activity Alliance (BOPPAA)

Lead Information

Name: Miss Emma Robinson

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Organisation: Barnsley Council

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Lead officer for this Eol:

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Expression of Interest

Lead organisation(s):

Barnsley Metropolitan Borough Council

Description of the intervention:

The Barnsley Older People Physical Activity Alliance (BOPPAA) is an innovative alliance of NHS, private, and third sector organisations offering physical activity opportunities for over 50s. BOPPAA was commissioned by Barnsley Council in response to consistently high local admission rates for falls and hip fractures. Injury from a fall is the most common cause of emergency hospital admissions for older people and falls are estimated to cost the NHS more than £2.3 billion per year. Further significant costs

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are associated with people who fall being unable to return to their own home, leading to the need for social care support, or discharge to a nursing or care home.

Helping older people to move more is a priority theme in Barnsley's new Health and Wellbeing Board Strategy, reflecting Barnsley's growing ageing population and a need to keep people physically and socially active for as long as possible to maintain health, wellbeing, and independence.

The BOPPAA alliance is led by Age UK Barnsley, and has over 85 member organisations delivering over 230 weekly strength and balance physical activities for older people (50+). The aims of BOPPAA are to:

- Increase provision of physical activity that improves strength and balance
- Improve emotional wellbeing and resilience
- Support independence
- Prevent falls in later life
- Reduce pressures within the health and care system

To meet the needs of the population and tackle health inequalities, Age UK Barnsley perform gap analysis to ensure equitable access to a diverse range of activities and targeting of specific groups where appropriate.

BOPPPA is funded via a grant from the Better Care Fund. The Better Care Programme spans both the NHS and local government to join up health and care services, so that people can manage their own health and wellbeing and live independently in their communities. BOPPAA is currently in the second year of a 3-year funded contract, with an option to extend for a further 2 years. This as an opportune time for evaluation to:

- Determine the success of BOPPAA in meeting its original aim of improving the strength and balance of older people to prevent falls
- Provide insight on the wider impacts of BOPPAA
- Assess whether the model has potential to be generalisable
- Provide timely evidence to inform future commissioning and funding arrangements

Additionally, the Chief Medical Officer's annual report (2023) highlighted the need to do more research with and for older people.

Geographical location(s) of the intervention:

BOPPAA covers the whole of the Barnsley Metropolitan Borough, in South Yorkshire. The aim is to deliver physical activity opportunities for older people in all six area council localities.

Current status and timescale(s) of the intervention:

We are currently in the second year of a three-year contract (ending March 2027), with the option to extend for a further two years should additional funding become available.

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

Funding arrangements have been jointly agreed by Barnsley Integrated Care Board (ICB) and Barnsley Council's Public Health and Communities Directorate. BOPPAA is funded through an annual £185,000 grant from the central government Better Care Fund (BCF). The BCF aims to support people in staying

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independent for as long as possible, reducing the need for hospital or long-term care.

Intervention Start Date:

1st October 2021 (start of pilot)

Intervention End Date:

31st March 2027 for the current 3-year funding contract with the option to extend for a further two years

Impacts:

Anticipated population health impacts

Increased strength & balance activity - many activities use the Later Life Functional Fitness MOT (FFMOT) tool. This includes 30 second chair stand, timed 8ft up and go, and handgrip strength.

Prevention of falls - monitoring of A&E admissions for falls and hip fractures

Improved mental wellbeing - BOPPAA uses the UCLA three-item loneliness scale and the Short Warwick Edinburgh Mental Wellbeing Scale.

Wider Impacts

Improved partnership working & collaboration - BOPPAA brings together a network of organisations

Upskilling of the workforce and volunteers - Training of staff and volunteers to deliver FFMOT, falls exercise and mental health first aid

Financial sustainability of alliance organisations - Income stream from paid activity sessions

Public/community involvement or engagement:

During Covid-19 and prior to developing BOPPAA, stakeholder engagement was undertaken with physical activity providers to explore opportunities that would support them to continue to operate. BOPPAA has a co-ordinated outreach and engagement programme performing FFMOT assessments at community events and groups. Older people are sign posted to activities that meets their needs in their community. Community events provide the opportunity for physical activity demonstrations and the chance to understand and listen to feedback from local people; this has led to the development of new physical activity groups.

There is continuous engagement with Member organisations through regular alliance meetings and a newsletter. Evaluation could aid further engagement with older people in Barnsley.

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

Representatives from key organisations such as Age UK Barnsley, BMBC, Healthy Bones, Barnsley U3A, Barnsley Premier Leisure, Barnsley Football Club Community Trust, and Barnsley GP Federation make up the 'core group' that meet on a regular basis to discuss referrals process, promotion, development of activities, training opportunities, and sharing of small grants information.

There is a wider BOPPAA 'network' of over 85 organisations that either deliver physical activity or provide the space for activities to take place. Many have been supported by the BOPPAA small grants

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programme to deliver more activities that increase strength and balance, with 67% becoming sustainable following the grants.

BOPPAA is also part of the disability, sport, and physical activity forum.

Weblinks:

Age UK Barnsley | Barnsley Older People Physical Activity Alliance -
<https://www.ageuk.org.uk/barnsley/our-services/barnsley-older-people-physical-activity-alliance-0d883597-c5fe-ed11-a81c-6045bd94e88e/>

Welcome to Age UK Barnsley - <https://boppaa.ageukbarnsley.org.uk/>

Learn more about the Barnsley Older People Physical Activity Alliance -
<https://www.youtube.com/watch?v=LvckoWVC2h0&t=1s>

Additional information:

In 2023, the BOPPAA was shortlisted in the Local Government Chronical awards within the Health and Social Care category.

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Name of intervention to be evaluated: SAFE (Safeguarding Adolescents from Exploitation)

Lead Information

Name: Mrs Amanda Baxtrem

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Organisation: Redcar & Cleveland Borough Council

Contact Information

Lead officer for intervention:

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Head of Service - Review resources and Vulnerable Young people

Childrens services

Redcar and Cleveland Borough Council

Second point of contact for this Eol:

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Resource Manager

Childrens Services

Redcar and Cleveland Borough Council

Supporting Director of Public Health (DPH) or Chief Executive Officer (CEO):

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Director of Public Health

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Expression of Interest

Lead organisation(s):

Redcar and Cleveland Borough Council

Description of the intervention:

Many families in our Borough live in fear in their communities, due to the extent of poverty & their generational alignment to organised crime. For some children and young people (C&YP), exploitation can appear to be the only life choice that is available.

Previous research by Salford university evidenced that the earlier we share information with C&YP, the greater the likelihood of children recognising exploitation and responding to it safely. The SAFE team have embraced this concept, working robustly to educate senior leaders, teaching & support staff across the Borough to educate and support C&YP about exploitation.

It is vital to build trusted relationships with C&YP before any meaningful & successful interventions can take place, so the SAFE team respond differently by thinking upstream about those C&YP who may be the next victims of exploitation. While the SAFE team hold responsibility for direct work with C&YP,

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SAFE are equally committed to awareness raising, working in partnership, educating others, supporting communities through trusted relationships/strengths based work, and changing perceptions/responses to at risk C&YP through our 'it takes a village' concept. We adopt an open, transparent & inviting culture to share our learning.

SAFE team have 3 primary work strands 1: direct work with children with over 40 different direct work tools, supporting individual child needs 2: delivering contextual safeguarding training across the partnership 3: robust prevention work offered to all education settings. We have produced multiple resources for C&YP & their families as part of our partnership & communication strategy. We deliver training to police, housing, local traders & target community hotspots through engagement with local counsellors. Our Education Specialist provides targeted interventions to small groups of children, now extended to 25/26 to education providers.

Partnerships/organisations in our Borough, and in other local authorities, have shown significant interest in what SAFE have created, why, and how this works. We are regional leaders around risk outside the home and developing responses through child protection work.

The SAFE team are 100% committed to our C&YP.

Geographical location(s) of the intervention:

Redcar & Cleveland

An area with variable demographic, rural and densely populated, post-industrial. High poverty levels & unemployment, highest levels of crime & serious violence in the county.

Current status and timescale(s) of the intervention:

SAFE is 12 months old and is currently active. In 2026, families of all Y6 pupil will receive an 'exploitation pack' raising knowledge, awareness of risk & how to keep their children safe.

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

Funding sources include Public Health, Department for Education & Staying Close. Current staff posts are currently fixed term until at least March 2027. Decisions on extending SAFE team funding until March 2027 will be known by the end of June 2026. Cost avoidance to date indicates that and once grant funding is exhausted Local Authority will continue to fund.

Intervention Start Date:

April 2024

Intervention End Date:

An end date is not anticipated, we recognise there is a need for this team to continue.

Impacts:

Incidents of self-harm have reduced, with improved well-being and an overall increase in child safety. Substance misuse & mental health have significantly reduced. C&YP are at less risk of death by misadventure from substance use.

Impact on the wider system includes a reduction in arrest/offending and serious violence rates, A&E attendance/admission, missing from home incidents. Disruption orders and property closure orders have provided safeguarding for C&YP & Communities. We have prevented children requiring multi agency response, partner intervention or admission to care.

Contextual Safeguarding training was delivered to over 2500 teaching/support staff across all schools/education providers in the borough. The Team have attended 48 community facing events.

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Public/community involvement or engagement:

Questionnaire feedback from C&YP/families/communities shows SAFE is impacting & creating a space & opportunities for children to thrive & be safe.

We are planning long term change to de-stigmatise language/culture & response to children through raising awareness & working on whole systems change to protect C&YP .

We would like PHIRST help to evaluate how we can do this and how we can involve our communities in this process: PHIRST to help us to evaluate how we have achieved change so far (process evaluation), what the outcomes are for C&YP/families & money saved to our funders & the wider system. We are interested in methods to involve our C&YP and communities in shaping SAFE going forward, via robust methods of consultation welcoming guidance & advice on methodology to achieve this.

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

Police and Community Safety Partnership, Youth Justice Service Health, Housing, Substance Misuse Service and Education are invested in this research opportunity.

SAFE team sits in RCBC Children's services

All work undertaken by SAFE is reported back to council scrutiny where decisions are made about any future funding, this is by a quarterly and annual report. On a Tees level we are further governed by the Tees Wide HOTH (Harm outside The Home) strategy reporting to the Safeguarding Children's Executive.

Weblinks:

<https://www.redcar-cleveland.gov.uk/children-and-families-services/safe>

<https://x.com/SAFEteamRCBC>

Additional information:

44 YP opened to SAFE; 17 successfully closed & a 48% reduction in Social Care threshold & significant reduction in harm. PHIRST to assist in identifying practice that may not currently be recognised to further enhance this positive trajectory. We have indicative cost avoidance but how do we translate that into actual evidence for partners and stakeholders so we can continue to share our effectiveness of our model on a regional and national basis.

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Name of intervention to be evaluated: Haringey's "Book your child's Vaccination" campaign

Lead Information

Name: Miss Angharad Shambler

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Organisation: Haringey Council

Contact Information

Lead officer for intervention:

Same details as lead applicant

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Senior Public Health Strategist- Health Protection & Population Health Improvement

Haringey Council

Lead officer for this Eol:

N/A- same as lead officer

Second point of contact for this Eol:

Daniel Subel, Daniel.Subel@nhs.net, Prevention and Vaccinations Programme Lead, Vaccination

Prevention Team, Chief Medical Officer and Place Directorate, NHS NCL ICB

Supporting Director of Public Health (DPH) or Chief Executive Officer (CEO):

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Expression of Interest

Lead organisation(s):

Haringey Council

Description of the intervention:

Haringey's "Book your child's vaccination" campaign was developed in response to significantly low childhood vaccination rates. Only 68% of children receive all required vaccinations by age five—well below the national target of 95%. The borough also experiences marked health inequalities, particularly among families where English is not the primary language. For example, vaccination uptake is 38% for Bulgarian-speaking and 56% for Romanian-speaking populations. Uptake also varies widely across ethnic groups, with notably low rates in Gypsy, Roma, Irish Traveller, and Black African and Caribbean communities.

The campaign aligns with key aims of the NHS vaccination strategy—specifically, improving access to childhood vaccinations and delivering them in local, accessible settings.

This intervention was designed to simplify the process of booking childhood immunisations and to clarify what vaccines are due at each age. A central component is an online form that parents can complete in

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under two minutes to request an appointment for their child. This form can be translated into multiple languages, making it accessible to a wider range of families.

The campaign launched on October 1st and has been promoted through:

- Health visitors (via new birth packs and letters to non-attenders),
- School readiness communications,
- Community-based engagement through family hubs, nurseries, leisure centres, and culturally relevant venues (e.g. Turkish restaurants, ABC Parents Club),
- Digital and print materials, including websites and newsletters.

This wide-reaching, multi-channel approach ensures that the campaign reaches parents through trusted points of contact in both health and community settings.

Geographical location(s) of the intervention:

Haringey, London (but with regional roll-out planned for 2026)

Current status and timescale(s) of the intervention:

The pilot started in October 2024 (initially via our 0-19 services) with wider roll out in the Haringey community from March 2025. Pilot to continue until 1st Jan 25 (then possibility of ICB roll out)

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

This campaign is funded via the public health grant and in kind resources from stakeholder partners (project management, leadership).

We have allocated roughly £10,000 per year to this intervention (mainly to fund the campaign design, print and advertising costs). There is also some additional contingency money available if necessary. There are no existing resources allocated to an evaluation, and it would be very challenging at this time to free up resource to be able to undertake a detailed evaluation. However, an evaluation is important for us to make a case for wider ICB roll out.

Intervention Start Date:

1st October 2024

Intervention End Date:

January 2026 (This would mean the pilot has reached over a year) and aligns with the major national immunisations schedule changes (for ICB roll out).

Impacts:

The anticipated impacts of this intervention are both practical and strategic:

- It educates parents about the timing and importance of immunisations.
- It simplifies the booking process, particularly for those who face barriers calling GP practices, by offering a digital alternative.
- It helps to reduce inequalities by offering multilingual support, removing language as a barrier to access.
- It opens up opportunities for earlier engagement with families around vaccine hesitancy, enabling more informed and confident decision-making.
- It is structured to be data-driven, with monthly review meetings to assess form uptake and parent feedback. This allows for continuous quality improvement and escalation of issues where needed.

Public/community involvement or engagement:

Community engagement has been integral to the development and delivery of the campaign. Initial

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feedback from parents at Haringey's children's centres was positive. Parents consistently reported that the online form was easy to complete, did not include unexpected questions, and was a preferred alternative to phoning their GP.

Throughout the pilot, further engagement has taken place through a follow-up process—where parents are contacted five days after submitting the form to assess whether they have received an appointment. This helps track outcomes and identify areas for improvement in real-time.

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

A dedicated task & finish group, was convened to support the programme's design and delivery.

Stakeholders include:

Core contributors (connected to evaluation):

- Haringey Start Well Clinical Lead and GP
- NHS NCL ICB Prevention and Vaccinations Programme Lead
- Senior Public Health Strategist, Haringey Council
- Public Health Communications Manager, Haringey Council

Wider partners:

- Early Years teams, children's centres, and family hubs
- CYP Commissioner
- 0–19 Health Visiting and School Nursing service
- Schools and education providers
- GP Federation
- Health Champions from underserved communities
- Other council departments

This wide-ranging collaboration ensures the intervention is rooted in community needs and supported across systems.

Weblinks:

Currently, there are no direct weblinks but we are working on a feature/press article on our local pages. The campaign and form is promoted through stakeholder and digital channels with specific campaign materials (digital, posters, leaflets) and corresponding QR code for the channel shared with stakeholders directly.

Additional information:

Evaluation can help to answer:

- Form users? Are they representative of underserved groups?
- Is this approach easier by parents/health professionals?
- Are children vaccinated earlier or on time?
- How many parents successfully received appointments? timescale?
- Engagement with vaccine hesitancy section: what are the insights?
- Conversion rate from campaign engagement to appointment booking?
- Is the campaign educating parents?
- When are parents available to take their children for vaccines?

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Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: Health Impact Assessment in Spatial Planning (HIASP)

Lead Information

Name: Mr John Wilcox

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Expression of Interest

Lead organisation(s):

Wakefield Council

Description of the intervention:

HIASP facilitates the use of Health Impact Assessment (HIA) in the built environment to create healthier housing and communities, improve and protect health and reduce inequalities.

Wakefield and Doncaster Councils developing and using bespoke tools to provide a framework to support house builders to carry out effective HIAs, that provide appropriate consideration of health impacts that may arise from housing development plans. The findings of the HIAs being used to inform public health planning comments, future revisions to the development proposals and ultimately any planning decision.

HIA is a “practical approach used to systematically judge the potential health effects of a policy, strategy, plan, programme or project” (WHO) that when applied in the planning system, “can help identify a set of evidence-based practical recommendations to promote and protect the health of local communities.” [Public Health England, 2020]

For HIASP HIA is taken to be a systematic, evidence-based approach including:

- a) identification of the health impacts a specific development proposal might have.
- b) assessment of the likelihood of those identified impacts occurring, which population they are likely to affect and their potential severity.
- c) identification of measures to minimise the effects of any negative impacts and maximise the effects of any positive impacts. Supporting the creation of developments that “do as much good and as little harm as possible.”

An HIA can cover a range of themes including access to and quality of green space, housing quality and amenities, and active travel infrastructure. The themes and questions explored in the two authorities' HIAs tools differ. This highlights the need to understand which approach is more effective, adaptable, and practical. Gaining this insight will be valuable for informing future practice.

HIASP process:

- A local validation checklist sets out if a HIA is required (based on set criteria, such as the size of the development). Planner requests house builder submit a HIA.
- HIA is completed by the house builder using a pro-forma HIA tool (Wakefield) or Microsoft Forms (Doncaster).
- Public health officers review HIA. Review includes quality assurance and health comments and recommendations.
- Health comments and recommendations are fed back to the house builder, planning officer and other decision makers (e.g. planning committee).
- Planning Officer utilises this information as part of their overall decision.

Geographical location(s) of the intervention:

The intervention takes place in the Wakefield District and Doncaster City Council planning authority areas.

Current status and timescale(s) of the intervention:

The intervention is ongoing and has been delivered for more than 9 years in both the Wakefield and Doncaster areas.

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Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

The intervention is an ongoing mainstreamed activity funded in both areas (Doncaster and Wakefield) through core public health and planning budgets. The HIA forms part of Local Plan policy in Wakefield and Doncaster and therefore is a long term commitment to undertake HIA's for both authorities.

Intervention Start Date:

2016 ongoing

Intervention End Date:

The intervention forms part of Local Plan policy in Wakefield and Doncaster and so is a long term commitment to undertake HIA's for both authorities.

Impacts:

Overall: New housing developments that improve and protect human health and contribute reduced inequalities.

Other: House builders effectively consider the health impacts of development proposals. As demonstrated by submission of good quality, evidence based HIAs.

Recommendations from HIA tools and/or public health comments impact planning process.

Public health/HIA recommendations realised as part of finished developments through highlighting the importance of health impacts on the end user and surrounding community.

Finished developments demonstrate characteristics of healthy place e.g. Physical activity opportunities, Active travel infrastructure & Accessible local services.

Better understand impact of HIA on built environment shaping regional and national policy and practice.

Public/community involvement or engagement:

Engagement has taken place with with planners, planning agents and house builders using HIA tool and as part of HIA tool development.

Significant engagement has also taken place with other local authorities through the Yorkshire and Humber HIA Evaluation group, part of the Y&H ADPH supported Healthy Places Col.

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

Wakefield Council

Doncaster Council

Weblinks:

Doncaster Council HIA Website

<https://www.doncaster.gov.uk/services/health-wellbeing/health-impact-assessments-hias-for-planning-and-development>

Wakefield Council HIA website

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<https://www.wakefield.gov.uk/health-and-advice/health-impact-assessment/>

Wakefield Tools:

<https://www.wakefield.gov.uk/media/h54nclpi/hia-planning-toolkit.pdf>

<https://www.wakefield.gov.uk/media/mdklnofl/hia-toolkit-non-residential.docx>

Additional information:

The use of HIA and greater public health input into spatial planning has long been promoted by bodies such as OHID and was part of the rationale for development of HIASP. However, there is still limited evidence around the impact of HIA in spatial planning and an evaluation such as this has the potential to impact on policy and practice nationwide.

Being able to demonstrate the impact HIASP is having will enable other authorities to make informed decisions around adoption of HIA based approach.

NIHR PHIRST local government expression of interest (Eol)

Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: Healthy Schools Knowsley

Lead Information

Name: Miss Sophie Aherne

Job Position: Public Health Programme Officer

Department: Public Health

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Organisation: Knowsley Metropolitan Borough Council

Contact Information

Lead officer for intervention:

Sophie Aherne

Lead officer for this Eol:

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Second point of contact for this Eol:

Jackie McMenamy

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Supporting Director of Public Health (DPH) or Chief Executive Officer (CEO):

Sarah McNulty- Sarah.McNulty@knowsley.gov.uk

Expression of Interest

Lead organisation(s):

Knowsley Council- Public Health

Description of the intervention:

Knowsley Council has recently published its Healthier Weight Strategy and has a strong focus on giving the best start in life.

Our childhood obesity levels are amongst the highest in the country, it is also the second most deprived local authority area nationally. With 46 of 98 neighbourhoods in the 10% most deprived. Once children in Knowsley reach the end of primary school almost 1 in 2 children are overweight/obese. Tracking Knowsley's National Child Measurement Programme (NCMP) data, it follows some concerning trends.

-83.5% of pupils who were obese in Reception in 17/18, were also obese in year 6 in 23/24.

-50% of those who were overweight in Reception in 17/18, became obese in year 6 in 23/24.

The cost of providing a school meal has increased recently due to the cost of living but government funding per meal has not kept pace. Meaning additional costs for meal provision being managed by school and council budgets whilst trying to keep costs for parents down. To mitigate this and realise some improvements to meals and school food environments a collaborative effort was introduced to provide financial incentive to schools on joining the Healthy Schools Knowsley programme.

The programme creates positive environments by adopting the healthy menu and promoting active lifestyles. Schools work towards bronze, silver and gold award by meeting certain criteria set. Launched

NIHR PHIRST local government expression of interest (Eol)

in November 2024, currently, 90% of Knowsley's 52 primary and SEND schools are engaged, with many achieving bronze. The council provides 47 of 52 primary schools meal provision, providing the perfect opportunity to influence healthier menus to a large population of children.

The intervention aims to reduce the number of children moving into higher weight categories in primary school through healthier food provision, education, and awareness. Secondary aims include maintaining a lower price per meal for parents paying for school meals, but receiving better quality food, as well as supporting schools financially to improve their school environment.

To date monitoring of the programme includes collecting data on schools engaging, attendance of staff training, school meal uptake and lunchbox swap stations implemented. Feedback is also gained from stakeholders collecting views, learning and experiences of the healthy school's programme to demonstrate its impact. Longer term 'healthy schools' will launch similar programmes across our early year's settings and secondary schools.

Geographical location(s) of the intervention:

Knowsley Primary and Special Schools

Current status and timescale(s) of the intervention:

Intervention began in November 2024. Schools aim to achieve each level of healthy schools within 6 a month period.

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

Funding of £0.250m for healthy school awards as a temporary investment from the Public Health Grant. There is no current budget for an evaluation.

Intervention Start Date:

November 2024

Intervention End Date:

July 2026

Impacts:

The intervention is designed to directly impact children's health by creating a healthier school environment, changing staffing behaviours, providing nutritious food, and monitoring obesity levels. By addressing health inequalities and influencing boarder social norms, the programme aims to foster long-term positive changes in children's health and well-being. Schools become places where healthy eating and active lifestyles are not just encouraged but are the standard, creating a cultural shift, making healthy living a natural part of daily student life. With the whole school approach implemented schools will have healthier pupils, presence, improved academic performance, behaviour and overall school success.

Public/community involvement or engagement:

Primary schools have began working on the Healthy Schools programme building engagement from pupils and parents. Engagement events such as assemblies, coffee mornings and online meetings have been held with pupils and parents throughout schools launch to promote healthy schools. The programme is also promoted on schools communication methods to parents through newsletters and websites. The programme has also been showcased on BBC Mersersyde radio, where one of our Knowsley headteachers spoke about the scheme and the benefits of its joined up approach between school meals and public health.

NIHR PHIRST local government expression of interest (Eol)

Some schools are running larger events to launch their 'Pledge for a Healthy and Active Future' these events invite pupils, parents, local organisations and health professionals.

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

Primary School staff- headteachers, learning mentors, class teachers, PE Leads

Primary School pupils

Parents and wider school community

Weblinks:

Additional information:

Seen as long term programme, but earliest ending 2 years as have at least funding for programme until then but the Healthy school programme and officer is permanent/ sustained beyond this.

NIHR PHIRST local government expression of interest (Eol)

Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: Provision of free loan cycles and training to Year 5 & 6 Pupils in the 10% most deprived Suffolk LSOAs

Lead Information

Name: Miss Emma Brinkley

Job Position: Project Manager - Natural Environment and Climate Change

Department: Public Health and Communities

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Organisation: Suffolk County Council

Contact Information

Lead officer for intervention:

Mr Christopher Grover, christopher.grover@suffolk.gov.uk, Behaviour Change Manager, Growth Highways and Infrastructure – Transport Strategy team, Suffolk County Council

Lead officer for this Eol:

Miss Emma Brinkley, emma.brinkley@suffolk.gov.uk, Project Manager - Natural Environment and Climate Change, Public Health and Communities – Place Making team, Suffolk County Council

Second point of contact for this Eol:

Miss Rosie Welch, rosie.welch@suffolk.gov.uk, Project Lead - Natural Environment and Climate Change, Public Health and Communities – Place Making team, Suffolk County Council

Supporting Director of Public Health (DPH) or Chief Executive Officer (CEO):

Stuart Keeble, stuart.keeble@suffolk.gov.uk, Director of Public Health, Public Health & Communities, Suffolk County Council

Expression of Interest

Lead organisation(s):

Suffolk County Council (SCC)

Description of the intervention:

Cycling is a key skill in the development of healthy, confident young people. It enables a level of independence at a young age, supports them to travel actively, increases their physical activity levels, reduces their risk of preventable ill physical and mental health, and benefits the environment through reduced carbon and air pollution emissions. Active travel for short journeys is also a key factor in determining whether people achieve the recommended 150 minutes of moderate activity per week for adults and 1 hour per day for children: 45% of active adults achieve this through active travel (Sport England).

The idea for this intervention came in Spring 2023 when Bikeability training was offered free of charge to a limited number of schools in the most deprived LSOAs in Suffolk (and 10% most deprived in England), because we know that people within this cohort are significantly less likely to meet the recommended levels of physical activity, most likely to be overweight or obese, least likely to be active, and experience

NIHR PHIRST local government expression of interest (Eol)

the greatest health inequalities.

Most schools refused this offer because not enough pupils had access to a cycle. In October 2023, a pilot scheme was developed where 31 loan cycles were provided to a school within a deprived Suffolk LSOA, actively targeted to year 5 and 6 pupils who did not have a cycle. When provided alongside the existing Bikeability training, there was a sustained increase in those who travel actively to school. This pilot scheme is still running.

To develop and expand the pilot, we now plan to provide 200-400 loan cycles to year 5 and 6 pupils in schools within or adjacent to the most deprived LSOAs in Suffolk. The cycles will be loaned at the start of year 5: pupils will retain them until they leave the school at the end of year 6, when they will be serviced and passed on to the incoming year 5s, creating a sustainable model. Alongside this, there will be free 'wraparound' skills training, including Learn To Ride and Bikeability Level 1 & 2 (pupils), Learn to Fix (parents) and Family Cycling Confidence (all).

Evaluation of this intervention would fill an existing evidence gap around the impact of Bikeability on health and wellbeing, which to date has not been addressed by the Bikeability Trust. Additionally, there has also not been a PHIRST evaluation in the East of England to date.

Geographical location(s) of the intervention:

The locations will be informed by the consenting schools within the 10% most deprived Suffolk LSOAs (IMD2019), particularly those within coastal communities, and adjacent LSOAs (where appropriate).

Current status and timescale(s) of the intervention:

The project is currently in the planning stages. Cycle delivery is expected to take place over winter 2025-26, with skills training starting in Spring 2026.

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

Loan cycles (200-400) and Learn to Fix are funded by the Active Travel England Capability Fund grant. Family and Cycling Confidence Sessions are funded by the Public Health Grant. The Bikeability training and Learn to Ride training is funded by The Bikeability Trust. Total funding allocated is £120,000.

SCC will undertake basic monitoring and evaluation of the rates of cycle use and travel to school patterns, with potential support from University of Suffolk Integrated Care Academy, subject to funding. PHIRST support could assist in expanding the scope and depth to evaluate wider HWB impacts

Intervention Start Date:

Oct 23

Cycles: Winter 2025/26

Skills: Spring 2026

Intervention End Date:

The intervention will continue for at least two years, at which point the cycles will be passed on to a new cohort of students.

Impacts:

We aim to increase physical activity and active travel levels among Year 5 & 6 pupils in Suffolk's most deprived communities by improving access to bikes and skills training (evidenced by numbers of pupils

NIHR PHIRST local government expression of interest (Eol)

walking/cycling to school and the number of daily/weekly minutes' activity). This initiative will boost school travel independence and reduce reliance on motorised transport.

According to the COM-B model, provision of skills training and loan cycles (and short-term loan of adult cycles if required) should generate lasting behaviour change within communities that are least physically active, most likely to be overweight or obese, and experience the greatest health inequalities, leading to improved health outcomes, reduced CO2 and local air pollution levels; and reduced NHS costs.

Public/community involvement or engagement:

Work was undertaken with the initial pilot school, directly with the school community and also with wider community groups local to the school. This led to direct financial support for ancillary items such as helmets and bike locks from the local Asda superstore and the local Rotary club.

Schools are best placed to engage with their local communities rather than the intervention appearing to be 'top down' from the local authority, and our engagement strategy is based on this: support the schools with time, advice, strategy and engagement material, but let the actual communication come from the school.

More generally, we have communicated with the public via SCC webpages and the local press (October 2023 - see weblinks section)

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

The extent of stakeholder involvement will be dependent upon the size of each school. Specific stakeholders will be identified once the consenting schools have been identified. However, these are likely to include (in addition to the schools themselves): Suffolk County Council, relevant district or borough council, local Councillors, Community Hubs, Family Hubs, Local Bike Shops (to support with purchasing and maintenance), relevant health groups, Active Suffolk, Active Travel England, local press.

Weblinks:

1. <https://www.lowestoftjournal.co.uk/news/23836688.new-bike-library-pilot-scheme-launches-lowestoft-school/>
2. <https://www.suffolk.gov.uk/council-and-democracy/council-news/bike-library-pilot-launched-at-lowestoft-school#:~:text=Suffolk%20County%20Council%20has%20today,Transport%20Strategy%20and%20Waste%2C%20said:>
3. <https://modeshift.org.uk/news/team-modeshift-national-sustainable-travel-awards-2024-winners-announced/>
4. <https://thewaytogosuffolk.org.uk/>

Additional information:

The pilot project won a National Sustainable Travel in November 2024 for Education Engagement Project of the year Team Modeshift National Sustainable Travel Awards 2024 Winners Announced! - Modeshift. A further related pilot is now undergoing trials with 'recycled' bicycles given to young adults leaving Suffolk County Council care.

NIHR PHIRST local government expression of interest (Eol)

Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: Substance Misuse Welfare Outreach Van

Lead Information

Name: Dr Rachel Chapman

Job Position: Consultant in Public Health

Department: Public Health

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Organisation: University Hospitals Coventry & Warwickshire NHS Trust

Contact Information

Lead officer for intervention:

Molly Cox

Partnership Manager

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Change Grow Live

1a Lamb Street, Coventry, CV1 4AE

Lead officer for this Eol:

Rachel Chapman

Consultant in Public Health

rachel.chapman@uhcw.nhs.uk

Joint appointment: Coventry City Council Public Health team / University Hospitals Coventry and Warwickshire NHS Trust

Second point of contact for this Eol:

Amander Allen

Substance Misuse Programme Manager

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Coventry City Council Public Health Team

Supporting Director of Public Health (DPH) or Chief Executive Officer (CEO):

Dr Allison Duggal

Director of Public Health

allison.duggal@coventry.gov.uk

Coventry City Council Public Health

Expression of Interest

Lead organisation(s):

Coventry City Council

Description of the intervention:

This intervention delivers targeted, community-based harm reduction and healthcare services to some of Coventry's most vulnerable populations, including individuals experiencing homelessness, street-based sex workers, and people who use drugs in high-prevalence areas of the city. These groups often face

NIHR PHIRST local government expression of interest (Eol)

significant barriers to accessing mainstream health services, including stigma, mobility issues, and lack of trust in institutions. As a result, they are at heightened risk of preventable health harms, including blood-borne viruses, wound infections, overdose, and untreated chronic conditions.

To address these disparities, we are launching a mobile outreach model using a fully equipped clinical van. This mobile unit will operate a minimum of twice weekly in partnership with Coventry City Council's Rough Sleepers Team and other local stakeholders. The van will provide low-threshold, trauma-informed care directly in the community, including wound care, naloxone distribution, safer injecting advice, blood borne virus (BBV) testing, contraception, and referrals to wider health and social services. The intervention will also serve as a vital point of contact for individuals who are otherwise disengaged from support systems, helping to build trust and pathways into longer-term care.

Experience within the substance misuse service finds that assertive outreach improves health outcomes and engagement among marginalised populations. By meeting people where they are; both geographically and in terms of readiness to engage, we can reduce harm, prevent crisis presentations to emergency services, and promote health equity.

Key Aims:

1. To reduce health-related harms among high-risk populations through accessible, community-based interventions.
2. To increase uptake of preventative health services, including BBV testing and wound care, and improve outcomes.
3. To strengthen multi-agency collaboration in addressing complex needs, particularly around homelessness and substance use.
4. To build trust and engagement with individuals who are often excluded from traditional healthcare pathways.
5. To reduce stigma associated with substance use and accessing support services

This intervention represents a compassionate, practical response to entrenched health inequalities, aligning with local public health priorities and national strategies on inclusion health.

Geographical location(s) of the intervention:

Within the local authority boundary of Coventry City Council, with specific locations to be identified based on levels of deprivation and unmet need.

Current status and timescale(s) of the intervention:

The outreach van has been purchased and is fully equipped. Relevant staff are currently being insured and schedules being created. We expect the first outreach to have taken place by 01/07/2025

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

The outreach van is funded as part of the core substance misuse service contract with Change Grow Live (CGL). This is a new service contract running from 2025-2030 (option to extend for 2+2 years). The service was redesigned following service user and stakeholder feedback.

The van was purchased via one off funding; a Rough Sleepers Supplementary Grant provided to the service, with maintenance and continuing costs being covered by the Substance Misuse Service core budget.

There are currently no resources allocated to an evaluation.

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Intervention Start Date:

01/07/2025

Intervention End Date:

31/03/2030 (funded as part of core service)

Impacts:

This intervention will improve population health by reducing preventable harms such as overdose, infections (eg BBV), and untreated chronic conditions among high-risk groups (inclusion health groups). By delivering care directly to underserved communities, it will narrow health inequalities, particularly for people experiencing homelessness and those engaged in street-based sex work.

Wider impacts include reduced pressure on emergency services, improved engagement with health and social care pathways, and stronger multi-agency collaboration. Subtle, non-clinical branding of the outreach van and materials will help reduce stigma, making services feel more approachable.

Ultimately, the intervention promotes a more inclusive, equitable health system.

Public/community involvement or engagement:

Better access to services was a key element within the service design of the new substance misuse contract, especially for vulnerable groups. Substance misuse service users were consulted as part of the design of the service specification (by Public Health) and the service model including outreach (by CGL). Barriers to community-based delivery, particularly access to private consultation rooms within primary care services have been experienced by CGL. The outreach van directly addresses this by offering a flexible, mobile provision.

Engagement continues through promotion of the van's availability to health and social care partners, including adult social care, housing, probation, and mental health services, alongside public-facing promotion via local media and social media channels.

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

Change Grow Live (service provider)

Coventry City Council (Public Health, Adult Social Care, Housing, Community Safety)

NHS services (Coventry and Warwickshire ICB, Primary Care, University Hospitals Coventry and Warwickshire, Coventry and Warwickshire Partnership Trust, West Mids Ambulance Service)

Community and Voluntary Sector partners (including Kairos WWT, Turnaround, Mind)

Weblinks:**Additional information:**

We would also be interested in including some form of economic evaluation if this is possible to help with assessment of value for money.

NIHR PHIRST local government expression of interest (Eol)

Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: Alcohol awareness within Wolverhampton's Punjabi community: liver health checks and supportive conversations on alcohol use

Lead Information

Name: Mrs Ellina Bawa

Job Position: Health Improvement Officer

Department: Public Health

Email/Phone: ellina.bawa@wolverhampton.gov.uk 07738103635

Organisation: City of Wolverhampton Council

Contact Information

Lead officer for intervention:

Name: Mrs Michelle Marie Smith

Job Title: Principal Public Health Specialist

Email: Michelle.Marie-Smith@wolverhampton.gov.uk

Department: Wolverhampton Public Health

Organisation: Wolverhampton City Council

Lead officer for this Eol:

Name: Mrs Ellina Bawa,

Job Title: Health Improvement Officer

Email: ellina.bawa@wolverhampton.gov.uk

Department: Wolverhampton Public Health

Organisation: Wolverhampton City Council

Supporting Director of Public Health (DPH) or Chief Executive Officer (CEO):

Name: John Denley

Job title: Director of Public Health

Email: John.Denley@wolverhampton.gov.uk

Department: Public Health

Organisation: Wolverhampton City Council

Expression of Interest

Lead organisation(s):

Wolverhampton City Council- Public Health Team

Description of the intervention:

This intervention offers liver health checks using fibroscanning and facilitates culturally sensitive conversations about alcohol in faith settings. Designed and delivered by people from the Punjabi community with lived experience, it aims to raise awareness of alcohol harm, support early diagnosis and treatment of liver disease.

Punjabi Sikh men in Wolverhampton experience disproportionately high rates of alcohol-related liver disease, hospital admissions, and deaths. One in eight (12.0%) residents identify as Sikh. Asian males in Wolverhampton are overrepresented in alcohol-specific deaths (31.3% of deaths vs. 21.7% of the

NIHR PHIRST local government expression of interest (Eol)

city's population) and in alcohol-specific emergency admissions (19.6% of admissions vs. 11.7% of its catchment population). This indicates unmet need in the Punjabi community and a systemic gap in engagement with prevention, early diagnosis, and treatment. Engagement shows previous alcohol interventions were not culturally appropriate and required a tailored approach. Fibro-scanning was identified as a tool to initiate conversations about alcohol, particularly with those who may not yet see their drinking as problematic. While liver disease is particularly high among Punjabi men in Wolverhampton, this intervention was designed to support the whole community, recognising alcohol-related harm can affect anyone. It aligns with national and local priorities to prevent alcohol-related harm and promote early action to prevent poor health outcomes. Fibroscanning can detect fibrosis of the liver at a subclinical stage and support early identification of harm in asymptomatic individuals. The intervention is led by Wolverhampton's drug and alcohol treatment services: Recovery Near You (RNY), and SUI (lived experience recovery organisation), and is co-delivered by community partners. In addition, RNY are working with liver specialists to review referral protocols to prioritise those most in need of specialist care while maintaining access for early support. From Nov 2024 to May 2025, 5 Gurdwaras and 1 Buddhist temple in Wolverhampton hosted community events. In this period, 393 people engaged with the service for a liver health check. Of these, 91 individuals were referred for further assessment. Due to demand, the service will continue running events 1-2 times per month to provide information and support in trusted community settings, by those with lived experience to connect, reduce shame and stigma, and foster open discussions.

Geographical location(s) of the intervention:

Wolverhampton

Current status and timescale(s) of the intervention:

In response to community demand the intervention will continue throughout the duration of the contractual arrangements for the provision of drug and alcohol services.

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

Wolverhampton's drug and alcohol services are commissioned by Public Health within the City of Wolverhampton Council using the Public Health Grant. Services are delivered by RNY and SUI under contracts awarded in 2017, active from 1st April 2018 to 30 September 2025. A new five-year contract will be directly awarded to the incumbent provider from 1 October 2025 under the Provider Selection Regime (Process C). Additional fibro-scanning capacity was supported through the OHID-funded Supplemental Substance Misuse Treatment and Recovery Grant (SSMTRG).

Intervention Start Date:

November 2024

Intervention End Date:

ongoing

Impacts:

Anticipated impacts include increased awareness of alcohol-related harm, improved service engagement, behaviour change, earlier diagnosis, reduced liver disease, fewer emergency admissions, and greater support for individuals and families. The intervention empowers communities and supports culturally appropriate delivery. This intervention shows strong potential to be widely generalisable across the UK. There is a lack of published studies evaluating liver health checks and brief alcohol interventions in non-clinical, culturally embedded settings. Several local authorities have expressed interest in

NIHR PHIRST local government expression of interest (Eol)

adopting this model, highlighting the need for timely evaluative research to inform future practice and local policy development.

Public/community involvement or engagement:

A workshop involving treatment services, community organisations, and the Punjabi community co-designed the intervention, identifying engagement barriers, emphasising the value of tangible tools like fibro-scans. Wolverhampton's drug and alcohol services then partnered with local faith centres, blending health expertise and lived experience to break down stigma, language, and cultural barriers. Relationships were built with Gurdwaras, central to the Punjabi Sikh community, where a lived experience worker engaged through seva (voluntary service) to build trust. Punjabi-language posters were displayed at Gurdwaras, which actively promoted events during prayers. Additional awareness was raised through local Punjabi radio promotions by RNY and SUIT, increasing community participation.

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

Wolverhampton's Drug and Alcohol Services: Recovery Near You (RNY) and Service User Involvement Team (SUIT) have both consented to being involved in the evaluation.

Weblinks:

<https://www.suitrecoverywolverhampton.com>

<https://www.recoverynearyou.org.uk>

https://www.instagram.com/suit_services

Additional information:

NIHR PHIRST local government expression of interest (Eol)

Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: #CovConnects Device Bank

Lead Information

Name: Mrs Laura Waller

Job Position: Digital Services and Inclusion Lead

Department: Customer Services

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Organisation: Coventry City Council

Contact Information

Lead officer for intervention:

Laura Waller

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Digital Services and Inclusion Lead, Customer Services, Coventry City Council

Lead officer for this Eol:

Laura Waller

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Digital Services and Inclusion Lead, Customer Services, Coventry City Council

Second point of contact for this Eol:

Valerie De Souza

valerie.desouza@coventry.gov.uk

Consultant in Public Health and Director of the Coventry Health Determinants Research Collaboration
Coventry City Council

Supporting Director of Public Health (DPH) or Chief Executive Officer (CEO):

Allison Duggal

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Director of Public Health and Wellbeing

Coventry City Council

Expression of Interest

Lead organisation(s):

Coventry City Council

Description of the intervention:

Populations already experiencing social and health inequalities are disproportionately vulnerable to digital exclusion, compounding their marginalisation. Social exclusion from structural determinants of health and limited access to technology, digital literacy, or connectivity exacerbates disparities in accessing healthcare services, information, and opportunities for health-promoting behaviours. As healthcare systems increasingly adopt digital tools, failure to address these disadvantages risks widening existing health inequities.

#CovConnects is a local authority-led digital inclusion programme, working in partnership with public,

NIHR PHIRST local government expression of interest (Eol)

private and Voluntary, Community, and Social Enterprise (VCSE) sectors, collaborating with local organisations to address barriers and leverage community strengths.

A central feature of #CovConnects is the Device Bank, which distributes devices and connectivity to local residents via VCSE organisations, council services, and NHS providers. These organisations undergo an online application process overseen by Senior Officers from key council services, including Public Health. The programme follows circular economy principles, reusing and refurbishing end-of-life council and NHS devices while securing funding and partnerships for distributing tailored equipment. To date, 4200 devices have been distributed, creating a sustainable local digital reuse and inclusion model. To support organisations receiving these kits, #CovConnects offers training sessions, engagement events, assistance with funding bids, and corporate volunteering opportunities, all designed to strengthen service and community capacity for impactful digital inclusion efforts.

#CovConnects has secured funding to develop social impact pathways, ensuring consistent and effective data recording to measure outcomes. In collaboration with Coventry's Health Determinants Research Collaboration (HDRC), the programme has recently introduced comprehensive mixed-method evaluation measures for Device Bank recipients to improve data collection and impact analysis. Together, these efforts focus on identifying priority groups and co-designing solutions aligned with public health objectives and broader social determinants of health.

Despite the growing emphasis on digital inclusion by the government and NHS, such initiatives remain under-researched, highlighting a valuable opportunity to analyse recorded data and assess its impact as a sustainable intervention.

Geographical location(s) of the intervention:

Coventry - A city-wide focus with specific attention to low IMD areas, including Stoke, Foleshill, St Michaels, Hillfields.

Current status and timescale(s) of the intervention:

Launched in June 2023 and is ongoing.

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

Coventry City Council core fund salaries required for the initiative via Customer Services, Public Health and Digital Services are approximated at ~£100,000 per annum (taking into account percentages of roles across delivery, strategic oversight and governance)

Devices are donated as end-of-corporate life via the council and NHS ICB Coventry and Warwickshire.

Mi-fi connectivity units secured via UK Shared Prosperity Fund @ £340,000

Department of Science, Innovation and Technology funding for social impact framework platform @£30,000 to be delivered by end of November 25

Intervention Start Date:

June 2023

Intervention End Date:

Ongoing

Impacts:

Previous research and evaluation has indicated broad impacts, whereby digital inclusion acts as an enabler for wider determinants of health.

1. Direct health impacts;

Reduction in digital exclusion, including its differential impact by social deprivation (low IMD areas and

NIHR PHIRST local government expression of interest (Eol)

Core20PLUS5 target communities).

Improved access to NHS services and broader support services (MIND, addiction supports), and associated health outcomes

2. Indirect health impacts.

Improved:

Access to essential services such as housing, universal credit

Social connection and communication

Digital skills and confidence

Wellbeing

Learning and development across education and skills

Financial inclusion

Employability

Access to transport, online shopping, banking and services

Empowerment, autonomy, quality of life

Public/community involvement or engagement:

#CovConnects is based upon the 100% Digital Leeds Community model for digital inclusion and health participation, therefore co-creation with communities is fundamental. Before launching Device Bank, multiple community-led pilots supported the initiative design, identifying the need for holistic support for organisations both throughout the application process and longer term, addressing support, skills, funding, and capacity challenges.

Regular feedback and engagement with community partners including Working Actively To Change Hillfields drives continuous improvements in devices, connectivity, impact measurement and delivery models. Developments such as 'rolling contracts' with Carers Trust, are co-designed directly with the organisation and needs of young carers as bespoke adaptations

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

- CCC Internal implementation teams: Customer Service, Public Health, IT, Digital Inclusion
- CCC Internal beneficiary teams: Employment and Skills, Libraries, Migration, Children's Services, Adult Social Care
- NHS: Device Bank recipients (such as Coventry Family Nurse Partnership, University Hospital Coventry and Warwickshire)
- NHS Coventry and Warwickshire Integrated Care Board and Health Inequalities programme
- ~150 VCSE as beneficiaries & public lead evaluation
- HDRC: Provides evaluation assistance and research expertise.
- University of Warwick
- WATCH: Additional evaluation of embedded Digital Health Advisor's role and impact of DB devices via HDRC
- Carer's Trust: Additional evaluation of DB- piloting standardised approach to distributing kit to young carers

Weblinks:

<https://www.coventry.gov.uk/cov-connects/device-bank>

<https://www.thebritishacademy.ac.uk/documents/5374/Co-producing-theory-of-change-evaluation-framework-digital-inclusion-programmes.pdf>

<https://www.janga.la/digital-lifelines>

<https://digitalinclusionkit.org/wp-content/uploads/2022/12/100-Digital-Leeds-model-for-a-community->

NIHR PHIRST local government expression of interest (Eol)

based-approach-to-digital-inclusion.pdf

Additional information:

Device Bank sits within a CRM (Customer Relationship Management system) enabling data capture of the intervention, identifiable to each organisation/initiative, including; organisation type, location (IMD/deprivation), communities of interest supported (20 categories), device type and quantity, intervention type, wider supports received (training, funding, etc). Surveys are available for both organisational and resident impact, alongside qualitative case studies, linked to the application.

NIHR PHIRST local government expression of interest (Eol)

Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: Community Wellbeing Champions Project

Lead Information

Name: Ms Nazira Vania

Job Position: Project Manager

Department: Public Health

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Organisation: Leicester City Council

Contact Information

Lead officer for intervention:

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Lead officer for this Eol:

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Second point of contact for this Eol:

Dr Katherine Packham, katherine.packham@leicester.gov.uk, Consultant, Public Health, Leicester City Council

Supporting Director of Public Health (DPH) or Chief Executive Officer (CEO):

Rob Howard, rob.howard@leicester.gov.uk, Director, Public Health, Leicester City Council

Expression of Interest

Lead organisation(s):

Leicester City Council

Description of the intervention:

The Community Wellbeing Champions (CWC) project is a public health-led intervention that works with voluntary, community, and social enterprise (VCSE) partners and other organisations and individuals trusted by communities to gain insight into health needs, enablers, and barriers and use it to shape and deliver health messaging, support, and services. The intervention aims to help achieve reductions in health inequalities through more targeted and effective engagement with people affected by poorer health access, experiences, and outcomes.

The project was implemented in 2021 to help reach communities during the Covid-19 pandemic, but recognising the unequal impact of the virus on the city reflected pre-existing disparities, it sought from the outset to support efforts to reduce health inequalities across the board. Using an asset-based approach, the project established the CWC Network, a partnership made up of trusted community organisations and figures. As causes of inequalities are nuanced and, especially in diverse areas, often further complicated by intersectionality, understanding them requires creative, unique, and inclusive approaches to engagement and research with affected communities, built on foundational trust and relationships that local authorities don't always have in place but which community organisations, etc. usually do. Working with them as community champions can help health messaging and support reach people who need it most more effectively and facilitate access to rich data and insight into the

NIHR PHIRST local government expression of interest (Eol)

differences, barriers, and 'micro-inequalities' faced by these groups, in turn informing more practical, meaningful, and impactful measures for tackling health inequalities.

One problem being addressed by this intervention is the absence or weakness of connection, trust, and relationships between the local authority and some communities – both newly emerging groups (e.g., through migration) and established populations that have historically been underheard or underserved. Another problem being addressed is a need for closer partnership working between public health and trusted organisations and individuals, essential for health interventions to be meaningful and impactful. A further problem being addressed is the need for more effective and timely ways to share information and resources and collaborate on health initiatives across organisations and sectors, which the CWC Network facilitates through numerous channels, activities, and schemes.

Geographical location(s) of the intervention:

Leicester (city)

Current status and timescale(s) of the intervention:

Current status is ongoing. The active period of the project (development and delivery of CWC Network) was around 2 years from October 2021, after which the intervention moved to mainstream business.

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

The intervention is funded by Leicester City Council's Public Health budget. The amount of funding allocated: approx. £150,000 per year. There are no resources currently allocated to an evaluation.

Intervention Start Date:

October 2021

Intervention End Date:

CWC has become a mainstream/business as usual part of the PH service. The project manager is permanent,. There is no expected end date.

Impacts:

CWC is an indirect health intervention, in that it seeks to strengthen relationships and establish foundations with community organisations and figures, upon which direct interventions on specific health inequalities can be built. The anticipated impact of this indirect intervention is therefore the development and delivery of better-informed and targeted direct health interventions. Our key performance indicators are based on monitoring outputs and activities (e.g., members, events) rather than capturing wider health outcomes. While feedback from members tells us CWC is beneficial to them and the communities they serve, an evaluation would help us understand to what extent and in what ways this intervention is beneficial and what impact it is having more generally on health inequalities.

Public/community involvement or engagement:

The public is engaged through various CWC Network member activities and events, such as attending health and wellbeing fairs in community centres and undertaking/supporting consultations. Learning and insight from this type of community engagement is used to inform and shape CWC work and resources. However, the public has not been directly involved in the design or delivery of the intervention (the CWC project) itself.

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes

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ahead? - Yes

Stakeholder involved:

Note: The CWC Network currently has around 300 members, so it is not practical to consult all members about the proposed evaluation and secure consent from prospective participants at this stage. Similarly, it is not practical to engage with and secure consent from people who access the services of CWC. However, the board previously expressed support for an evaluation, so we are confident sufficient numbers and types of stakeholders will consent to being involved if it goes ahead and have ticked 'yes' to the consent question based on this assumption.

- VCSE partners and other network members ('champions')
- People who access services/support from champions
- General public
- Public Health teams/colleagues
- Wider local authority services/teams/colleagues
- Wider health sector colleagues

Weblinks:

n/a

Additional information:

n/a

NIHR PHIRST local government expression of interest (Eol)

Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: Sexual and Reproductive Health community provision

Lead Information

Name: Mrs Mobola Jaiyesimi

Job Position: Public Health Strategist

Department: Public Health

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Organisation: London Borough of Bexley

Contact Information

Lead officer for intervention:

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Lead officer for this Eol:

Mobola Jaiyesimi; Mobola.Jaiyesimi@Bexley.gov.uk; Public Health Strategist; Public Health; Bexley Council

Second point of contact for this Eol:

Tara Piasetski; Tara.Piasetski@bexley.gov.uk; Public Health Consultant; Public Health; Bexley Council

Supporting Director of Public Health (DPH) or Chief Executive Officer (CEO):

Nicole Klynman; Nicole.Klynman@bexley.gov.uk; Director of Public Health; Public Health; Bexley Council

Expression of Interest

Lead organisation(s):

London Borough of Bexley

Description of the intervention:

Background

The London Borough of Bexley commissions a community-based sexual and reproductive health (SRH) service to address the diverse and evolving needs of its population. This initiative responds to local health inequalities, limited access to SRH services among vulnerable groups, and the need for a coordinated, evidence-based approach to sexual health promotion. A key challenge is that there is no level 2 or 3 SRH clinic available within the Bexley geographical boundary, therefore limiting access to services for Bexley residents. The community based offer is expected to help bridge the gap and signpost residents to available services.

Rationale

The intervention is designed to improve SRH outcomes by delivering a high-quality, accessible, and responsive service. It aims to reduce barriers to care, particularly for underrepresented and disadvantaged populations, and to align with national policy and best practice. The service will be adaptable to emerging trends and evidence, ensuring relevance and effectiveness over time.

NIHR PHIRST local government expression of interest (Eoi)

Key Aims

1. Access and Equity - Ensure all residents of all ages, especially vulnerable groups, have access to SRH education, testing, and support.
2. Health Promotion - Deliver targeted outreach and education, including condom distribution (via the pan-London C-Card Scheme) and chlamydia screening for young people under 16.
3. Digital Engagement - Maintain and enhance a user-friendly website to provide up-to-date SRH information and service access.
4. Capacity Building - Train frontline staff using the Making Every Contact Count (MECC) approach to embed SRH promotion across services.
5. System Integration - Collaborate with wider partners to align with SRH strategies and developments.
6. Innovation and Responsiveness - Empower the provider to implement evidence-based innovations and adapt services in response to changing needs and policy shifts.

Geographical location(s) of the intervention:

London Borough of Bexley

Current status and timescale(s) of the intervention:

On going contract, but can review one year of activity.

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

The programme is funded through the public health grant. The programme costs the council approximately £150,000. There are no further resources allocated to an evaluation.

Intervention Start Date:

1st October 2024

Intervention End Date:

30th September 2027

Impacts:

- Improved SRH outcomes across all age groups through increased access to education, testing, and preventative services.
- Early detection and treatment of STIs, leading to reduced transmission rates and long-term health complications.
- Reduction in disparities by targeting services to vulnerable and underserved populations
- Empowerment of disadvantaged communities through culturally sensitive and inclusive service delivery.
- Strengthened local health system through workforce development (e.g., MECC training for frontline staff).
- Enhanced digital engagement and service visibility via an updated, accessible website.
- Contribution to broader public health goals, including reduced unplanned pregnancies and improved mental and emotional wellbeing related to sexual health.

Public/community involvement or engagement:

There has been limited public/community involvement

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes

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ahead? - Yes

Stakeholder involved:

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Weblinks:

<https://www.sexualhealthbexley.co.uk/>

Additional information:

N/A

NIHR PHIRST local government expression of interest (Eol)

Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: Breaking the Habit: Evaluating a Novel School- and Community-Based Youth Vaping Cessation Programme in Slough

Lead Information

Name: Dr Janet Ige

Job Position: Senior Public Health intelligence, insight and research manager

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Organisation: Slough Borough Council

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Slough Borough Council

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Expression of Interest

Lead organisation(s):

Slough Borough Council

Description of the intervention:

Slough's Stop Vaping Service is a novel, local authority-led vaping cessation programme for young people aged 12–18. Launched in October 2024 and delivered through the Integrated Health and Wellbeing Service (IHWS), the 6–12 week intervention is fully funded through the Public Health Grant. As one of the first targeted youth vaping cessation initiatives in England, it responds to a growing public

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health and equity concern.

Youth vaping has been nationally recognised as a major issue, prompting the UK Government to introduce a ban on disposable vapes from 2025. Around 18% of 11–17-year-olds have tried vaping and nearly 9% vape regularly, raising concerns about addiction and long-term cognitive harm. Locally, a 2023 mixed-methods study with 879 secondary students and follow-up focus groups revealed high awareness but underreporting of vaping due to fear and stigma. Young people described school environments as punitive rather than supportive, shaping the intervention's non-punitive, confidential, and youth-centred approach.

The findings were discussed with the PSHE Network and Headteachers' Forum, helping to co-design the intervention with schools. Multiple referral pathways were built in, including self-referral via SMS, school staff referrals, and identification through routine safeguarding conversations. The programme is currently running in one college and two secondary schools.

Intervention Components

- Weekly Behavioural support sessions over 6–12 weeks with trained advisors.
 - Age-appropriate cessation aids provided where needed.
 - Very Brief Advice (VBA) training for at least 50 professionals, including teachers and youth workers
- The programme reflects evidence that multi-component approaches combining behavioural, structural, and educational support are most effective for youth vaping cessation. It is also informed by the COM-B model.

Programme Aims

- Reduce youth vaping prevalence through tailored behavioural and therapeutic support.
- Equip professionals to identify and refer young vapers through targeted training.
- Address health inequalities by engaging underserved groups and reducing stigma.
- Generate scalable insights through independent evaluation to inform future policy and practice.

Since launch, the programme has gained national attention including BBC coverage and growing interest from other local authorities. A formal evaluation would provide vital evidence on its effectiveness, acceptability, equity impact, and scalability.

Geographical location(s) of the intervention:

Delivered in

Langley College, Langley Ward

St Joseph's Catholic School, Wexham Court

Beechwood School, Britwell Ward

Britwell is ranked the most deprived ward in Slough, with higher rates of poverty

Current status and timescale(s) of the intervention:

The intervention is ongoing, launched in October 2024 during the final year of the previous IHWS contract, and has been embedded into the recommissioned service from April 2025 for a five-year period.

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

The intervention is funded through the local authority's Public Health Grant as part of the Integrated Health and Wellbeing Service (IHWS), which receives £700,000 annually. It was introduced in the final year of the previous IHWS contract and has been embedded into the recommissioned service from April 2025 for five years. While no specific funding is allocated for evaluation, internal resources such as stakeholder engagement support, analyst support and contract monitoring systems are in place, with PHIRST being explored to support a more comprehensive evaluation.

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Intervention Start Date:

October 2024

Intervention End Date:

The intervention will be funded until 31 March 2030, underscoring the value of evaluation in measuring impact and informing delivery model adjustments

Impacts:

The intervention is expected to improve population health by reducing vaping prevalence and nicotine dependence among young people in Slough. The programme addresses health inequalities by targeting young people at greater risk of vaping-related harms, with delivery focused in areas of high deprivation to address local health disparities. Beyond cessation, the intervention is already showing positive wider impacts. These include improving staff confidence in addressing the broader social and emotional challenges linked to vaping, and creating stronger links between education, community services, and public health teams. Early data show that 54 YP have enrolled, with 26 successfully quitting vaping, 2 non-quits. We have a data monitoring system to track outcomes, referrals, and engagement

Public/community involvement or engagement:

Public and community involvement has been integral to the intervention from the outset. Insight from a borough-wide youth study shaped the intervention's supportive, non-punitive approach. Findings were shared with education and community stakeholders, including the PSHE Network and Headteachers' Forum, enabling co-design of referral pathways and integration into school settings.

Frontline professionals such as teachers, youth workers, and school nurses continue to inform training and delivery. A mystery shopper exercise with a local youth charity further assessed the service's accessibility and acceptability, informing improvements to communication and engagement.

A dedicated PPI group of young people from youth organisations will be established to shape the evaluation

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

Slough Borough Council (Public Health) Commissioner and funder of the intervention.

SBC (Trading Standards) to share insight on disposable vape ban

Solutions4Health- Delivers the Stop Vaping Service as part of the Healthier Slough integrated wellbeing service

Leads at participating Schools and Colleges (Langley College and Beechwood School) Support referral pathways and delivery.

PSHE Network and Headteachers' Forum –Involved in early co-design and stakeholder engagement.

Together as One Local Youth Charity – Delivered the focus group discussion and mystery shopper project assessing service accessibility. They will facilitate access to young people

Young People (12–18) – Primary beneficiaries and contributors to insight and future evaluation. To be accessed via Together As One and provider.

Weblinks:

<https://healthierslough.co.uk/>

<https://www.bbc.co.uk/news/articles/cqxwd0e3r5jo>

<https://www.slough.gov.uk/news/article/442/new-stop-vaping-service-for-teens-now-available-in-slough>

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<https://thelink.slough.gov.uk/news/slough-youth-vaping-cessation-press-release>

<https://www.sloughfamilyservices.org.uk/kb5/sloughcst/directory/service.page?id=p-miXDuu5Q>

<https://ecigator.com/news/slough-free-vaping-cessation-teenagers/>

Additional information:

Slough's Stop Vaping Service is among the first targeted youth vaping cessation programmes in England, combining behavioural support, NRT, and VBA training across schools and youth settings. The intervention has attracted national interest, with presentations delivered at OHID West midlands (100+ local authorities) and OHID Southeast webinar.

This growing interest highlights the need for a robust, independent evaluation. PHIRST support would provide evidence on effectiveness and equity impact

NIHR PHIRST local government expression of interest (Eol)

Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: Stepping Stones

Lead Information

Name: Dr Owen Richardson

Job Position: Data and Insight Enablement Lead for Public Health

Department: Data and Insight

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Organisation: Kirklees Council

Contact Information

Lead officer for intervention:

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Lead officer for this Eol:

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Public Health Specialist, Wakefield Council

Supporting Director of Public Health (DPH) or Chief Executive Officer (CEO):

Rachel Spencer-Henshall

rachel.spencer-henshall@kirklees.gov.uk

Deputy Chief Executive and Executive Director for Public Health and Corporate Resources (DPH),

Kirklees Council

Expression of Interest

Lead organisation(s):

Leeds Mind

Description of the intervention:

Stepping Stones is a post-crisis support service for adults (18+ years) in Calderdale, Kirklees and Wakefield (CKW) who have attended A&E having self-harmed or attempted suicide. It provides trauma-informed peer support; working with clients, their family and friends to build tools to aid recovery, reducing the likelihood of future self-harm or suicide attempts.

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Main objectives are to contribute to:

- A reduction in suicides across CKW
- Improved support for people who self-harm
- Improved support for people who attempt to take their own life

The service was commissioned in 2024 by CKW Local Authorities (Public Health) and led by Leeds Mind, as a proof of concept to address the gap in provision for post crisis services. The peer-led service is unique to CKW, using lived experience to inform and enhance delivery.

Rational:

Suicide audits across CKW identified gaps in post-attempt care and a lack of consistent, evidence-informed interventions. These findings, combined with reports of missed opportunities to identify non-fatal suicide attempts and self harm, made it difficult to understand the scale of need or plan effective responses. Audits also showed a significant number of people who die by suicide have previous suicide attempts and/or self-harm, making this a key service area to build upon. The service was developed to respond to these challenges, aligning with the West Yorkshire Health and Care Partnership's Ten Big Ambitions' priority to reduce suicide across the region. A commitment to compassionate, data-informed and locally responsive suicide prevention services reflects local strategies.

Stepping Stones aims to:

- Address the high risk of repeat self-harm and suicide attempts, providing trauma-informed peer support interventions for 6-10 1:1 sessions
- Reduce barriers to mental health care – addressing stigma, social isolation or cultural barriers that make engaging with mainstream services difficult
- Deliver person-centred, post-crisis support – After initial hospital discharge or crisis intervention, individuals can struggle to find appropriate ongoing care, leaving them isolated
- Align with best practice –designed in line with NICE guidelines; integrated into local suicide prevention strategies
- Address gaps in family/social network involvement – those lacking support systems need strategies to engage and strengthen their networks for lasting recovery
- Build local evidence of suicide attempt causes and understanding which interventions work

Geographical location(s) of the intervention:

Calderdale, Kirklees and Wakefield

Current status and timescale(s) of the intervention:

Following a mobilisation period of 3 months from April 2024, the service began in July 2024. It is currently active and funded to March 2026.

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

Stepping Stones is funded by Kirklees, Wakefield and Calderdale Local Authority's and is delivered by Leeds Mind. The current amount of funding for delivery of the two year contract is £213,825 per annum. As part of commission Leeds Mind are required to deliver monitoring and outcomes data. There is no resource allocated for formalised, external evaluation.

Intervention Start Date:

July 2024

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Intervention End Date:

March 2026

Impacts:

Anticipated impacts are as follows:

- Population Health: Evidence that the service is contributing to a reduction in suicides across CKW
- Health Inequalities: Improve access to services for underserved groups, reducing disparities in suicide risk, tailoring interventions to local needs, co-producing services with lived experience, and tracking long-term mental health outcomes to ensure sustained engagement and effectiveness.

Public/community involvement or engagement:

Stepping Stones was co-produced with people with lived experience of self-harm and suicide attempts. During mobilisation, focus groups were conducted with Leeds Mind's clients, particularly those Suicide Bereavement and suicide prevention programmes, to shape the service model. A steering group, made up primarily of people with lived experience works with Leeds Mind to refine the service, consider its impact and ensure the effectiveness of delivery.

- Engagement with local networks: The team is active within regional suicide prevention forums across Calderdale, Kirklees, and Wakefield, ensuring collaboration across NHS, public health etc.
- Partnerships with other local VCSE organisations create a joined-up approach and assuring effective signposting to complementary services.

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

- Commissioners – Public Health teams in Calderdale, Kirklees, and Wakefield.
- A&E Departments & Mental Health Liaison Teams – Key referral routes from local hospitals.
- Service Users & Their Social Networks – Adults accessing support and those involved in their recovery.
- Local VCSE Organisations – Partners offering complementary support (Touchstone, MESMAC, Healthy Minds, etc.).
- Suicide Prevention Networks – SPAN, SPAG, SPOG forums.
- GPs & Social Prescribers – Supporting long-term care pathways.
- Leeds Mind Staff – Practitioners with lived experience, delivering peer support.

Weblinks:

<https://www.leedsmind.org.uk/services/stepping-stones/>

Additional information:

Kirklees Council has committed to increasing its evaluation activity through recruitment of a new Research and Evaluation Officer (REO), part-funded by the NIHR LARP programme. The REO will be available to support the PHIRST team, and will hope to use this project as a learning opportunity for sharing best practice approaches to evaluation with the wider organisation.

NIHR PHIRST local government expression of interest (Eol)

Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: Universal offer of free physical activity in leisure centres through Be Active

Lead Information

Name: Miss Rhona Duff

Job Position:

Department: Birmingham City Council

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Organisation: Birmingham City Council

Contact Information

Lead officer for intervention:

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Lead officer for this Eol:

As above

Second point of contact for this Eol:

Ibrahim Subdurally-Plon, Ibrahim.Subdurally-Plon@Birmingham.gov.uk, Public Health Service Lead, Public Health Division, Birmingham City Council

Supporting Director of Public Health (DPH) or Chief Executive Officer (CEO):

Sally Burns, Sally.Burns@birmingham.gov.uk, Director of Public Health, Public Health Division, Birmingham City Council

Expression of Interest

Lead organisation(s):

Birmingham City Council

Description of the intervention:

Rationale and Background

The Be Active (BA) service is operated by the Sport and Leisure team at Birmingham City Council (BCC). Launched in 2009, the programme provides free access to leisure centres and wellbeing facilities for all Birmingham residents. It offers a variety of physical activities, including swimming, gym sessions, group exercise classes, and fitness programmes.

The service was created to address health inequalities by removing financial barriers to participation. By offering free, localised access to physical activity, BA encourages greater community involvement and supports healthier lifestyles.

Currently, the BA service is available in 19 leisure centres across Birmingham, primarily located in areas with higher levels of deprivation and physical inactivity. This ensures that communities most in need have the opportunity to engage in regular physical activity.

BCC directly manages eight leisure centres, while also partnering with two commercial operators through funding agreements:

NIHR PHIRST local government expression of interest (Eol)

- Birmingham Community Leisure Trust (BCLT) – 9 centres
- Places for People (PfP) – 2 centres

The eight BCC-run centres contribute 35,000 hours of free physical activity annually, accounting for 81% of the total BA provision.

Some centres also offer inclusive classes for individuals with disabilities, as well as age-specific and gender-specific sessions, such as women-only or men-only gym times and classes.

Service Aims

The Be Active programme supports residents in meeting the Chief Medical Officer's physical activity guidelines and aligns with the goals of the Birmingham Joint Health and Wellbeing Strategy, particularly the "Active at Every Age and Ability" initiative. Key objectives include:

- Reducing the percentage of physically inactive adults to below 20% by 2030
- Raising physical activity levels among children and young people to meet the national average by 2030
- Halving the inactivity gap among ethnic communities
- Halving the inactivity gap between people with and without disabilities by 2030

The programme collects both process and impact data, with support from Public Health, to monitor progress and outcomes.

Due to upcoming funding changes, the service is undergoing a redesign. The findings from this evaluation will play a crucial role in shaping the future direction of the Be Active programme.

Geographical location(s) of the intervention:

The focus of this evaluation is on the eight BCC run leisure sites, based in Perry Barr, Handsworth, Nechells, Shard End, Kingstanding, Bordersley Green, Small Heath and Winston Green.

Current status and timescale(s) of the intervention:

The intervention is operational and funded until 2027.

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

The intervention is funded by both the Council General Fund and the Public Health Ring Fenced Grant. The Public Health team have set up a Memorandum of Understanding (MOU) with the service to ensure capture of impact data pertaining to the service.

Intervention Start Date:

2009

Intervention End Date:

31st March 2027

Impacts:

The intervention aims to increase physical activity levels in a largely inactive population. We know more deprived communities are less physically active, and by focussing the offer in these areas, we intend to reduce health inequalities associated with physical inactivity.

Public/community involvement or engagement:

The service has started to carry out some service user feedback as part their MOU quarterly return for 2024/2025. 95% of respondents said they either agreed or strongly agreed that they were satisfied with the service. 94% of respondents said they either agreed or strongly agreed that they would recommend the service to others. This data can be made available to the evaluation team.

NIHR PHIRST local government expression of interest (Eol)

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

BCC Public Health

BCC Sport and Leisure Service

Weblinks:

https://www.birmingham.gov.uk/info/50356/creating_an_active_birmingham/2990/birmingham_wellbeing_service

Additional information:

NIHR PHIRST local government expression of interest (Eol)

Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: Targeted Specialist Stop Smoking Service in Hartlepool, which aims to address health inequalities.

Lead Information

Name: Mrs Claire Robinson

Job Position: Public Health Principal

Department: Public Health

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Organisation: Hartlepool Borough Council

Contact Information

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Advanced Public Health Practitioner

Public Health

Hartlepool Borough Council

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Hartlepool Borough Council

Second point of contact for this Eol:

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Expression of Interest

Lead organisation(s):

Hartlepool Borough Council

NIHR PHIRST local government expression of interest (Eol)

Description of the intervention:

Hartlepool's smoking rates for adults 18+ in 2023 were at 13.5%, which is higher than the average for England at 11.6% and the North East at 11%. Smoking attributable hospital admissions in Hartlepool in 2017/19 were reported at 2365 per 100,000, in comparison to 1398 per 100,000 for England. Historically support for stop smoking was provided through Community Navigators who offered behavioural support and provision of the Swap to Stop Scheme. North Tees & Hartlepool NHS Trust have been commissioned to enhance the delivery of an evidence-based service in line with national policy. The services aim to improve long term quit rates at 4 weeks, 12 weeks, 6 & 12 months, the data is broken down by ward level. Collaborative working enhances the current provision of behavioural support and vape offer (12 week vape bundle delivered to service user directly by taking the offer to the individual), through settings the individual is already accessing such as community hubs, workplaces, drug and alcohol services, social housing, primary care network social prescribers and community pharmacy. The service contributes to reducing health inequalities by targeting the following priority groups: routine and manual workers, people with diagnosed mental health or long-term conditions, pregnant smokers, drugs and/or alcohol users, smokers who have a planned admission to hospital, smokers eligible for NHS health checks and residing in areas with Index of Multiple Deprivation (IMD) score 1-3, and smokers accessing housing services.

Service users are actively signposted to other lifestyle services and support based upon their initial assessment and the service is open and accessible to support those who may require multiple quit attempts. The service facilitates access to pharmacotherapy via prescription and other supply routes such as a voucher scheme. The service offers pop up clinics to access other targeted groups, such as in workplaces and within social housing provision. The service has a centralised data management system and provides quarterly data to Hartlepool Borough Council, which includes demographics, health indicators and background on the need for targeted support.

We are aware of similar evaluations completed by PHIRST in other areas of the country but there are no evaluations in the North East. Evaluation of our intervention will offer more insights for other local authorities on community outreach and flexible support options for underserved groups

Geographical location(s) of the intervention:

Targeted community settings across Hartlepool in ward areas with highest IMD. Hartlepool's IMD shows a deprivation score of 35, the second highest in the North East and 10th highest in England.

Current status and timescale(s) of the intervention:

The contract has been running since October 2024 and is funded to run until March 2027. Data that has already been collected to date, will also be able to be used for evaluation.

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

Hartlepool Borough Council was awarded £142,389 annually (for 5 years) to enhance stop smoking services. The Stop Smoking Service is a Public Health commissioned service with a contract term until 31st March 2027, funded by the Department of Health and Social Care

Intervention Start Date:

October 2024

Intervention End Date:

March 2027

Impacts:

NIHR PHIRST local government expression of interest (Eol)

Using available data collected by Hartlepool Borough Council we would like to know how successful this specialist service is in reducing smoking rates and targeting these priority groups in order to reduce health inequalities. Our goal is to find out whether the service helps people within these target groups to stop smoking and the methods which they are using to stop smoking, it is also likely to tell us whether the service is reaching the priority groups and reducing preventable differences in health outcomes. If the evaluation shows the service is successful in accessing targeted groups, other local authorities could model this to reduce health inequalities further across the country. The evaluation could also allow us to develop the service further and secure further funding.

Public/community involvement or engagement:

To inform the service design, HBC conducted a public online survey, gathering valuable input. As part of a previous application for funding we have worked with a member of the public (Mish Loraine). Additionally, as part of a previous application for funding we engaged with a Lived Experience Advisory Group (LEAG). Four members of the LEAG provided feedback. Two were ex-smokers, one current smoker and one who had transitioned to vaping. Two had previously used stop smoking services. All four represented one or more of the target priority groups including diagnosed mental health or long term conditions and planned admission to hospital. The group are committed to supporting an evaluation of the intervention (with appropriate remuneration).

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

Professor Catherine Houghton (Academic at Northumbria University with experience in evaluating smoking interventions)
Professor Jo Gray (Academic at Northumbria University with expertise in health economics)
Dr Angela Rodrigues (Academic at Northumbria University with experience in evaluating smoking interventions)
Dr Sophie Orton (Academic at University of Nottingham with expertise in smoking during pregnancy)
Claire Robinson - Public Health Principal HBC
Ashley Musgrave - Advanced Public Health Practitioner HBC
Melissa Pinder - Advanced Public Health Practitioner HBC
Donna Mason - Service Manager (Specialist Smoking Service)
Paul Crawshaw (Academic at Teesside University with expertise in public policy)
Christian Morgner - (Academic at University of Sheffield with expertise in EDI)

Weblinks:

Smoking Needs Assessment Hartlepool -
https://www.hartlepool.gov.uk/downloads/download/2481/smoking_needs_assessment_2023
Fingertips Smoking Data - Hartlepool - <https://fingertips.phe.org.uk/search/smoking>

Additional information:

The stakeholders listed above have previously worked together on a NIHR Three Research Schools Prevention Research Programme Practice Evaluation Scheme Application, which unfortunately was unsuccessful. The stakeholders are fully supportive of our application to PHIRST.

NIHR PHIRST local government expression of interest (Eol)

Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: Project W - Women's Substance Misuse Service Evaluation

Lead Information

Name: Miss Katie Wood

Job Position: Public health development Manager

Department: Public Health

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Organisation: Portsmouth City Council

Contact Information

Lead officer for intervention:

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Ambition Portsmouth Service Manager

Intuitive Thinking Skills

Lead officer for this Eol:

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Research Development Lead

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Supporting Director of Public Health (DPH) or Chief Executive Officer (CEO):

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Director of Public Health

Portsmouth City Council

Expression of Interest

Lead organisation(s):

Ambition Portsmouth

Portsmouth City Council

The Society of St James

Description of the intervention:

In Portsmouth there are approximately 1900 people in substance misuse treatment with just 33% being women, this means that our recovery services are predominantly attended by men.

NIHR PHIRST local government expression of interest (Eol)

Often, women living with substance use difficulties have a complex combination of needs and competing priorities. Their lives can be chaotic, and they are significantly more likely than men to have experienced violence - either in childhood or through historic or active domestic abuse, having a compounding impact on their lives.

For those with childcare responsibilities, especially sole caregivers, there may be logistical difficulties in attending appointments.

For those who have experienced abuse and violence, a mixed sex setting may feel intimidating and sometimes triggering, especially where there may be a risk of continued abuse and exploitation from others accessing the service.

In December 2023 Portsmouth's Public Health team completed a women substance misuse needs assessment this involved focus groups and interviews with 13 female service users and seven professionals working with women with complex needs.

A quote from one woman :

"The recovery hub [community substance misuse service] is so intimidating... I go exactly the time I need to. I put my headphones in and keep my head down until my person comes over."

One recommendation from the needs assessment was to have more women-only services or groups.

The Ambition Portsmouth team (our commissioned lived experience service) offering the use of their building for one day a week to pilot a woman only provision, they made the commitment that they would get their male staff to work elsewhere in the city.

In July 2024 an inclusive, welcoming and safe space for women was opened at every Thursday, 10am - 4pm.

Project W is for woman with any substance use need and/or history of rough sleeping.

The strategic aim of Project W is to provide a woman only safe space to increase access for women to recovery treatment and support, and to support women to achieve their recovery outcomes.

This complex intervention is delivered by a range of partners and offers women access to:

- Help and support with alcohol and substance misuse
- On-site family and recovery workers
- On-site childcare to enable attendance
- Weekly Freedom programme/domestic abuse support
- Weekly activities such as arts and crafts and acupuncture
- Mental and physical healthcare
- Support from peer mentors
- Money, debt and finding employment advice

Geographical location(s) of the intervention:

The project support Portsmouth Women - living in the PO1-PO6 Postcode

Current status and timescale(s) of the intervention:

The service is currently operational and currently runs two days a week (Wednesday and Thursdays). The Thursday was launched in July 2024 the Wednesday was launched in February 2025.

Availability of funding:

NIHR PHIRST local government expression of interest (Eol)

Has all required funding to deliver the intervention been secured? - Yes

Funding:

The project has relied on the good will of services for example Stop Domestic Abuse Service provide staff to deliver the Freedom programme and healthy relationship workshop.

There is small amount of grant funding for the project. In 2025/2026 £18,000 from the Drug and alcohol treatment and improvement recovery grant (DATRIG) has been made available to support the Projects running costs, including onsite childcare and creche.

Intervention Start Date:

July 2024 - there is currently no end date.

Intervention End Date:

An evaluation can start at any time - There is no end date

Impacts:

The number of women in treatment in Portsmouth has increased since the start of the project. The numbers attending the service have grown steadily and reached a peak of 24 women in one day. Women are reporting they are much more comfortable in a women's-only environment.

A survey with 22 women attending found: - 100% had felt safe accessing, 100% have made positive changes in their life , 95% had improved self-reported mental health and wellbeing, 85% had accessed support for their substance use

Women stated the following :

"I love that it's a safe place. I can access multiple things in one and get support."

"It's confidential and has really helped me. It's made me feel more confident. I would recommend to a friend because hopefully it would help them in the same way."

Public/community involvement or engagement:

An engagement survey was launched to find out what women wanted from the service, several meetings with key stakeholders and women with lived experience were held. The survey had 79 responses. Women were asked what type of provision they would like to see, day/time of the week, any barriers accessing, the service name.

Women wanted to see a range of services being offered, the most requested service (78% of women) was mental health support, this was followed by one-to-one support from key worker (66%).

20% of women said childcare would be barrier. The women have taken ownership of the service. One women recently released from prison on licence, obtained a food hygiene qualification working in the prison kitchen; each week she leads on preparing the lunch for the service.

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

Fran Brooker - Ambition Portsmouth

Sue Woodhouse Stop domestic abuse

Tay Kilty - Society of St James

Mary Scogings - Uni City Medical Centra

Nancy Fellows - Creatful

NIHR PHIRST local government expression of interest (Eol)

Humly Creche
Lynne Doel - Portsmouth City council

Weblinks:

<https://www.portsmouthrecovery.org/advice-resources/>

Additional information:

Several discrete elements of the project that could be considered for evaluated include, understand the uptake, reach and impact of a Women only space on the aims of the project. Understand the uptake, reach and impact of peer support on the aims of the project . Understand who is not accessing the project,; such as women with children subject to Child protection plans.

NIHR PHIRST local government expression of interest (Eol)

Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: Healthy You: an evaluation of an all age, weight neutral (WN), psychologically informed community weight management (WM)/lifestyle support service

Lead Information

Name: Mrs Ruth Everson

Job Position: Public Health Manager

Department: Public Health, Health and Adult Services

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Organisation: North Yorkshire Council

Contact Information

Lead officer for intervention:

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Lead officer for this Eol:

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Second point of contact for this Eol:

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Public Health Manager

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North Yorkshire Council

Supporting Director of Public Health (DPH) or Chief Executive Officer (CEO):

Louise Wallace

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Director of Public Health (Health and Adult Services)

North Yorkshire Council

Expression of Interest

Lead organisation(s):

North Yorkshire Council

Description of the intervention:

The intervention:

NIHR PHIRST local government expression of interest (Eol)

The intervention will focus on implementing a new weight management service (WMS) model for North Yorkshire (NY). The new model was developed after a long process of insight-gathering among local professionals, partners & the public.

Aims:

- Provide an all-age WMS across NY, to support children & adults around long-term lifestyle behaviour change, resulting in a reduction in levels of obesity.
- Provide a person-centred & flexible service offering a range of options (e.g. group sessions, 121s, face to face, online, physical activity) that reflects the complexities of life & the impact of living with health conditions.
- Reduce health inequalities & increase access to the service for underrepresented groups e.g. men, SMI.
- Implement & evaluate new approaches to providing WMS e.g. using behavioural science, WN & psychologically informed approaches.
- Inform the future commissioning of WMS.

Background & rationale:

As obesity rates continue to rise, LAs are tasked with developing evidence-based, effective & appropriate approaches to the commissioning of local community WMS. Over the past 10 years NY has had a range of WMS pilots & commissioned services for children, young people & adults. Community WMS provision has evolved over this period, maintaining delivery against NICE guidance & evidence-based practice, local needs & priorities. In preparation for both the adult & families service contracts coming to an end (Dec 2024), there was a desire to consider a redesign of the service, including the application of WN, compassionate & psychological approaches.

Psychologically informed working & weight neutral approaches:

The British Psychological Society recommends the incorporation of psychologically informed practice within WMS (1), & WN approaches (focusing on wellness rather than weight) may be more likely to result in long-term behaviour change & improved health (2). This reflects findings from our own insight work; innate psychological issues can contribute to people's struggles with food & weight. By incorporating these approaches into the WMS we hope to improve effectiveness of sustained weight loss.

Why this is important:

Psychologically informed & WN approaches are under researched with limited evidence implemented across England. The evaluation could contribute to wider academic learning on effective implementation. There is much interest from academic colleagues @gional forums on the effectiveness of such approaches.

Geographical location(s) of the intervention:

Delivered across NY, the service is provided specifically from community venues in each of the seven county localities (Craven, Hambleton, Harrogate, Richmondshire, Ryedale, Scarborough & Selby).

Current status and timescale(s) of the intervention:

The new service model was implemented on 1 January 2025 and has no defined end date.

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

£425k is allocated annually 25/26 & 26/27.

NIHR PHIRST local government expression of interest (Eol)

Local Government Reorganisation (April 23) created an opportunity to consider measures other than externally commissioning WM provision. NY Public Health entered an SLA with the Council's Sport & Active Well-Being (SAW) service to deliver the new WMS from 1 January 25. In-house delivery aligns with new SAW delivery model as per a recent local Strategic Leisure Review. Working with SAW, we understand the extent to which the changes in WM approaches can offer flexible, more creative programmes when supporting people with other health conditions.

Intervention Start Date:

1 January 2025

Intervention End Date:

No confirmed end date but Public Health funding likely to be up to the end of December 2026.

Impacts:

The new model has been developed to facilitate participation & effective LT behaviour change for those experiencing health inequalities. Our greatest obesity levels are in areas of deprivation & inclusion groups. A person-centred approach will allow individual circumstances to be considered (e.g. carers/family of people living with disabilities). The service being psychologically informed will enable the emotional & psychological needs of individuals, especially those experiencing inequalities to be considered in the provision of their support. There is much learning that can be achieved, through this evaluation, to influence the design of other wellbeing services related to obesity impacts (MSK, CVD, cancer, diabetes); the opportunity to provide significant evidence wider than the WMS.

Public/community involvement or engagement:

Engagement with adults was undertaken (May – July 2023) to understand why individuals and population groups were not accessing the existing WMS, despite being eligible. It helped to ensure future community WMS provision in NY met the requirements of individuals & communities with the greatest need. Therefore, it is timely to consider the impact of that feedback.

In addition, the family WMS ('Healthy Families') was piloted from March 2023-December 2024. Feedback was sought from families around their experience of the service & its impact. It helped to identify what had supported families most, what didn't work & any recommendations for improvements. The findings of these pieces of engagement can be provided upon request.

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

Stakeholders connected to the intervention include:
Healthy You clients (adults, children & families)
Healthy You service provider (SAW team, North Yorkshire Council)
Commissioners of tier 3 WM provision (Humber & NY ICB)
Tier 3 service providers (commissioned by the ICB)

There is already a procedure in place for GDPR and obtaining consent from clients. When clients agree to receive the service, they agree to having their details stored for service monitoring purposes. If we would be sharing their data with a third party, we would obtain their consent for this at the point of need.

Service providers, SAW & ICB colleagues are fully engaged & consent to the evaluation proposal.

NIHR PHIRST local government expression of interest (Eol)

Weblinks:

An overview of the new WMS offer:

<https://www.northyorks.gov.uk/healthy-living/healthy-you>

Access to WMS annual reports:

<https://www.nypartnerships.org.uk/healthyweight>

References from main body of the application:

(1)

<https://cms.bps.org.uk/sites/default/files/2022-08/Psychological%20Perspectives%20on%20Obesity%20-%20Addressing%20Policy%2C%20Practice%2C%20and%20Research%20Priorities.pdf>

(2) <https://pubmed.ncbi.nlm.nih.gov/35819360/>

Additional information:

The development of this new, innovative model came in conjunction with local government re-organisation (LGR) in NY. This benefited the transformation of the new WMS and influenced things such as how & where delivery takes place. The findings of the evaluation will be particularly helpful to disseminate to local authorities looking to transform their own WMS, especially those who are planned to go through LGR.

NIHR PHIRST local government expression of interest (Eol)

Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: Nottingham and Nottinghamshire Smoking and Tobacco Control Alliance: partnership for bringing about system change to reduce smoking prevalence.

Lead Information

Name: Mr Nathan Smith

Job Position: Public Health commissioning Manager

Department: Public Health

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Organisation: Nottinghamshire County Council

Contact Information

Lead officer for intervention:

Nathan smith, nathan.smith@nottscc.gov.uk, Public Health Commissioning Manager, Public Health and Communities, Nottinghamshire County Council

Lead officer for this Eol:

Nathan smith, nathan.smith@nottscc.gov.uk, Public Health Commissioning Manager, Public Health and Communities, Nottinghamshire County Council

Second point of contact for this Eol:

Gita Rana, gita.rana@nottinghamcity.gov.uk, Public Health Manager – Smoking and Tobacco Control, Public Health, Nottingham City Council

Supporting Director of Public Health (DPH) or Chief Executive Officer (CEO):

Vivienne Robbins, Vivienne.Robbins@nottscc.gov.uk, Director of Public Health and Communities, Public Health and Communities, Nottinghamshire County Council

Expression of Interest

Lead organisation(s):

Nottinghamshire County Council and Nottingham City Council

Description of the intervention:

Smoking continues to be the top cause of preventable illness and early death in the UK and one of the main causes of health inequalities in England. England's previous Tobacco Control Plans have stated support for the development of local partnerships led by local authorities and bringing together health, local government and voluntary sector partners, on the basis that this can lead to consistent and aligned public health campaigns, integrated action, improved referral pathways between local services, and effective targeting. 70% of local authorities have a Tobacco Control Alliance and whilst there are plenty of published research evaluations of tobacco control interventions there is limited research that evaluates the effectiveness of a Tobacco Control Alliance to bring about change in whole system approaches and improve population health.

The Tobacco Control Alliance aims to eliminate tobacco-related harm to create a smoke-free generation for Nottingham and Nottinghamshire by 2040. As we come to the end of our first three-year delivery plan

NIHR PHIRST local government expression of interest (Eol)

and look forward to the next three years, now is an appropriate time to evaluate the effectiveness of the Alliance and to understand the processes, relationships and partnership dynamics that interact to bring about change in whole systems approaches and contribute to the reduction in health inequalities. The objectives of the Alliance are to reduce smoking prevalence through concerted, partnership action, increasing and joining up activities, enhancing communications to increase uptake of services, and thus demonstrably improve health in the long term by reducing smoking-related diseases. The tangible health outcome of the Alliance activity is therefore reduced smoking prevalence within the local partnership area.

The Alliance is well established with 29 members, a formal governance structure, agreed vision and action plan, terms of reference and record-keeping of actions, therefore it will be feasible to evaluate its effectiveness to bring about change in whole systems approaches.

Evaluating the effectiveness of the Alliance will provide essential evidence for improving operations to help reduce the harms on health caused by smoking. We can understand the effectiveness of this approach in achieving system change and thus improved outcomes on tobacco control and smoking for residents, which would be generalisable to the wider England population due to the prevalence of tobacco control alliances across the country.

Geographical location(s) of the intervention:

The alliance covers the entirety of Nottingham City and Nottinghamshire County.

Current status and timescale(s) of the intervention:

The alliance has been running for three years and currently in the process of launching its second three-year delivery plan.

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

The Alliance does not have funding as a partnership body; its individual members all have funding of their own. The resources held by the lead partners include Public Health grant assigned to smoking & tobacco control and additional funding associated with the Smokefree Generation proposals since 2024. There are no existing monetary resources allocated to an evaluation, if we were successful in application, public health teams working on this agenda would lean in as appropriate and we would ask alliance members to support through involvement with the PHIRST team and evaluation as appropriate.

Intervention Start Date:

Launch of Alliance May 2023.

Intervention End Date:

Delivery plan 2 runs from 1.4.2025 until 31.3.2028. We would aim to complete the evaluation and present the results by the end of 2026.

Impacts:

The anticipated impact is to increase effective collaboration at a system level, to increase the power of smoking cessation interventions in terms of reach and impact, by enabling integrated action and effective targeting. This in turn would lead ultimately to the reduction of health inequalities and improvement of population health.

- Identifying specific interventions which would not have happened without the Alliance. Such as a harm reduction pathway, patient safety where there is medical oxygen use in households.
- The integration of tobacco control across the wider operations of the health and care sector
- Identifying any areas where the approach has led to inhibition of potential action.

NIHR PHIRST local government expression of interest (Eol)

- Strengthening community and individual capacities
- Tackling structural inequalities

Public/community involvement or engagement:

1. Stakeholder engagement to develop of the Alliance vision and strategy. Continuous stakeholder engagement through the formal governance structure, alongside wider engagement through social media platforms and links with other partnership bodies (e.g presentations to local respiratory network) helps to ensure others can influence relevant activities.
2. Public engagement takes place through commissioned behavioural insight work with the local community. This included small focus groups homing in on emerging issues of public concern such as youth vaping. Behavioural insight work has helped shape priorities within the second delivery plan.
3. Information is gathered through JSNA on population need, specific intervention evaluations which include service user and stakeholder feedback.

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

Notts County Council
Nottingham City Council
Sherwood Forest Hospital Trust
Nottingham University Hospital
Nottinghamshire Healthcare Trust
Doncaster and Bassetlaw Hospital Trust
Notts and Nottingham ICB
South Yorkshire ICB
Your Health Notts
Thriving Nottingham
Ashfield District Council
Bassetlaw District Council
Broxtowe Borough Council
Gedling Borough Council
Mansfield District Council
Newark & Sherwood District Council
Rushcliffe District Council
Notts Fire and Rescue Service
Notts Police
University of Nottingham

Weblinks:

www.nottsmoking.co.uk

Additional information:

NIHR PHIRST local government expression of interest (Eol)

Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: Whole Systems Approach to Healthy Diet, Weight and Obesity
A NI regional project using 6 early adopter sites – phased approach

Request to extend original NIHR successful application: NIHR135936

Lead Information

Name: Mr David Tumilty

Job Position: Health & Social Wellbeing Improvement Senior Manager

Department: Health Improvement

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Organisation: Public Health Agency

Contact Information

Lead officer for intervention:

David Tumilty

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Health & Social Wellbeing Improvement Manager

Health Improvement Department

Northern Ireland Public Health Agency

Second point of contact for this Eol:

Hannah McCourt

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Health & Social Wellbeing Improvement Manager

Health Improvement Department

Northern Ireland Public Health Agency

Supporting Director of Public Health (DPH) or Chief Executive Officer (CEO):

Dr Joanne McClean

Joanne.McClean@hscni.net

Director of Public Health

Northern Ireland Public Health Agency

Expression of Interest

Lead organisation(s):

Consortium bid obo local Councils across NI led by NI Public Health Agency

Description of the intervention:

We are applying to extend the current research (NIHR135936) to provide an opportunity to research implementation of phases 3,4,5&6 of the PHE model. We feel that alot of excellent work and learning has occurred to date and it would be extremely beneficial to extend the learning through the remaining phases.

Whole System Approaches look at the cause and effect relationship of all the factors contributing to obesity.

NIHR PHIRST local government expression of interest (Eol)

In line with the 2018 Public Health England (PHE) guidance, it is ROPIG's hope that the Community Planning function will provide the means to facilitate collaborative gain to help address this important public health issue.

Background

Overweight and obesity is a major problem in Northern Ireland. 26% children and 65% of adults are living with overweight or obesity.

People living with obesity are at a higher risk of a range of life-threatening diseases.

Overweight and obesity is a complex issue and it is now well accepted that obesity results from many interconnected policy, environmental, social, economic, cultural, behavioural, and biological causes.

Strategic Context

The PHA is tasked, by the Department of Health, to lead the implementation of a "A Fitter Future for All" 10-year framework to prevent and address overweight and obesity.

In addition, it is linked to a number of other policies and strategies including; Programme for Government, Making Life Better – Public Health Framework for N. Ireland, Community Planning, Active Living – Sport and Physical Activity Strategy, NI Food Strategy etc.

In addition, a new Obesity Prevention Strategy is in development for NI, to replace 'A fitter Future for All - 2012-2022' & includes plans regarding WSA to Obesity. This is supplemented by the Institute of Public Health Evidence Synthesis of WSAs to Obesity, launched on 26th January 2023, which provides an overview of the evidence and important factors to ensure success. These have been factored into the decision-making process with early adopter sites and will be instrumental in driving plans in NI.

Three of the six early adopter sites have started working through the WSA phases i.e. phase 1 & 2 and have started phase 3 in 2025. We want to build on the implementation work to date and gather further information to provide research that will help us learn from our approach and share important insights into whole system approach work.

Geographical location(s) of the intervention:

Six Northern Ireland Councils

Primary focus will be on sites 1,2 & 3, with the learning being applied to sites 4,5&6.

Current status and timescale(s) of the intervention:

Ards&North Down, phase 3

Belfast, phase 3

Derry & Strabane District, starting phase 2

Antrim & Newtownabbey Borough, phase 1

Armagh, Banbridge&Craigavon, phase 1

Causeway Coast&Glens, commence 2025

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

Project is supported through the community planning process with state of readiness assessed for each Council and commitment already given by key partners to give staff time and resources to support the initiative.

Agreement is also secured locally with the health trusts

Financial allocation will be agreed once the parameters of the project are established and the lead

NIHR PHIRST local government expression of interest (Eol)

organisations identified in line with local action plan development e.g. in DC&SDC a part time coordinator recruited to drive forward this work.

No specific evaluation resources have been allocated to the initiative at present.

Intervention Start Date:

ANDBC (January 2023)

Intervention End Date:

WSA is an ongoing process. With learning to date, any new Early adopter site would be expected to implement phases 1-6 in a 24 month period.

Impacts:

EARLY IMPACTS:

- o Increase awareness of WSA to Obesity
- o Ownership & collaboration i.e. Informed and engaged stakeholders
- o Stakeholders understand & integrate Whole Systems thinking to practice
- o Early adopter sites learn from each other as WSA is implemented.
- o Evaluation / measurement framework developed re impact/outcomes

EARLY PERFORMANCE MEASURES:

- o Launch & implement WSA
- o #stakeholder events held
- o #people trained in WSA
- o #meetings held to drive WSA forward
- o X6 WSA Cause & effect maps developed
- o Develop x6 action plans
- o Knowledge & understanding of WSA pre & post engagement.
- o WSA integration into community planning action plans
- o Learning community established
- o # Learning Community meetings & lessons learnt
- o Process evaluation completed to support and share learning

Public/community involvement or engagement:

The WSA to Obesity was selected, through Belfast's Community Plan to be a pilot site for testing the new Co-design Framework. This work started in early 2025 and we plan to use the framework to embed co-design in this work, from phase 3 onwards.

We have also engaged with a number of people with lived experience of overweight and obesity, to ensure we are incorporating the correct terminology and language, and to help develop the most suitable name for this work.

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

Patricia Mackey (A&NDBC)
Necole Donaghy (Belfast City Council)
Edel O'Doherty (Developing Health Communities)

Weblinks:

https://assets.publishing.service.gov.uk/government/uploads/system/uploads/attachment_data/file/82

NIHR PHIRST local government expression of interest (Eol)

0783/Whole_systems_approach_to_obesity_guide.pdf

<https://www.health-ni.gov.uk/news/health-survey-ni-first-results-201920>

<https://www.health-ni.gov.uk/sites/default/files/publications/dhssps/obesity-fitter-future-framework-ni-2012-22.pdf>

<https://www.gov.uk/government/publications/systems-thinking-for-civil-servants/journey>

Additional information:

In NI, the Director of Public Health is not based in Councils, therefore this is unique. DPH in NI is based in the Regional Public Health Agency for NI.

The expertise of PHIRST CONNECT would be best suited to continue evaluating implementation of the WSA in NI.

NIHR PHIRST local government expression of interest (Eol)

Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: North Somerset Healthy Workplaces Programme

Lead Information

Name: Ms Liz Green

Job Position: Food, Nutrition & Workplace Health Manager

Department: Health Improvement, Public Health

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Organisation: North Somerset Council

Contact Information

Lead officer for intervention:

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Lead officer for this Eol:

As above

Second point of contact for this Eol:

Georgie MacArthur, Consultant in Public Health, Health Improvement, Healthy & Sustainable Communities, North Somerset Council

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Supporting Director of Public Health (DPH) or Chief Executive Officer (CEO):

Matt Lenny, Director of Public Health, Healthy & Sustainable Communities Directorate, North Somerset Council

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Expression of Interest

Lead organisation(s):

North Somerset Council

Description of the intervention:

The Healthy Workplaces Programme is a free public health initiative supporting employer-led staff health and wellbeing programmes in North Somerset. It offers a flexible framework based on six healthy workplace goals and nine wellbeing topics. Participating workplaces work towards bronze, silver, and gold awards, receiving resources, support, and access to annual events.

The programme aims to improve the health of the working-age population, benefiting local businesses through reduced sickness absence, improved employee satisfaction, increased productivity, and enhanced reputation. It also supports recruitment, retention, and networking, while extending the reach

NIHR PHIRST local government expression of interest (Eol)

of local health services such as smoke-free support, physical activity initiatives, and healthy lifestyle programmes.

Evidence shows that good work positively impacts health. In the UK, 131 million working days are lost annually to sickness, with musculoskeletal and mental health issues being the top causes. One in three employees with long-term conditions haven't discussed them with their employer. Around 300,000 people aged 16 - 64 leave the workforce each year due to work-limiting conditions, and are three times less likely to return to employment. In North Somerset, 74.1% of employed individuals report a physical or mental long-term health condition - higher than the regional (69%) and national (65.3%) averages - with 19% reporting musculoskeletal issues.

Local workplace accreditation schemes have proven to be effective in promoting workplace health. The Healthy Workplaces Programme is based on this model, incorporating NICE guidance and insights from other local authorities.

Since its pilot phase - shaped by six initial workplaces - the programme has grown significantly. As of 2023, 23 workplaces representing 28,571 employees have joined. To date, five bronze and one silver award have been granted.

A pilot cardiovascular health check initiative, funded and delivered between November 2024 and May 2025, further strengthened workplace relationships. It involved 52 workplaces and we delivered 1,196 health checks.

We would like to build on this strong foundation to understand the impact of the programme on health and wellbeing outcomes and health inequalities in our population.

Geographical location(s) of the intervention:

North Somerset

Current status and timescale(s) of the intervention:

Ongoing programme

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

The Healthy Workplaces Programme is funded as part of the Public Health grant funding within the council. This allows us to employ staff to run the programme and non-salary funds are mainly used for communications and events activities.

Annual non-salary budget 2023-2024 = £17,000

Intervention Start Date:

Development 2020, pilot 2021-22, launched March 23

Intervention End Date:

ongoing

Impacts:

We aim to reduce health inequalities and improve working-age health by targeting employers in routine and manual roles. North Somerset faces significant inequality, with five areas in Weston-super-Mare

NIHR PHIRST local government expression of interest (Eol)

among the most deprived 5% nationally and a 12.2-year life expectancy gap between the most and least affluent male populations. The region includes rural and coastal towns, such as Weston-super-Mare, which experience high coastal deprivation and distinct employment patterns - lower employment growth and more jobs in hospitality, tourism and social care (Breaking Barriers Innovations, 2024). Additionally, 17% of workers earn below two-thirds of the UK median hourly pay (The Health Foundation, 2020). We apply proportionate universalism, focusing more on employers in the most deprived areas.

Public/community involvement or engagement:

The programme has utilised community feedback from the Health and Wellbeing Strategy and North Somerset Physical Activity Strategy engagement to inform actions within the programme e.g. through design and delivery of a physical activity funding scheme for workplaces, where 10 workplaces applied for funding with 8 being awarded funding for physical activity initiatives within the workplace. The projects are due to be completed in October 2025. We undertake regular engagement events (5 per year) for employers in North Somerset, request feedback from events, and undertake an annual survey of Members of the Healthy Workplaces Programme.

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

Stakeholders include the Healthy Workplaces Team, wider input from across the council services like the Economy team, Transport team, Healthy Lifestyle team, Physical activity team, Smokefree team, as well as businesses across North Somerset, their Health & Wellbeing leads, Human Resources and those offering support to workplaces e.g. Mental health support, financial wellbeing support and other organisations.

The Healthy Workplaces team will be fully involved in the evaluation and we have a lot of engaged Members of the Healthy Workplaces programme who can be invited to contribute and am confident that they will be engaged with the work.

Weblinks:

<https://www.betterhealthns.co.uk/professionals/healthyworkplaces/north-somerset-healthy-workplace-award-programme/>

<https://www.betterhealthns.co.uk/professionals/healthyworkplaces/>

<https://www.betterhealthns.co.uk/professionals/healthyworkplaces/north-somerset-workplace-cardiovascular-health-check-programme/>

Additional information:

NIHR PHIRST local government expression of interest (Eoi)

Call Title: 25/107 PHIRST LA Eoi March 2025

Name of intervention to be evaluated: Healthy Cornwall Stop Smoking Service

Lead Information

Name: Mr MARC NEELD

Job Position: Public Health Practitioner

Department: Wellbeing & Public Health

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Organisation: Cornwall Council

Contact Information

Lead officer for intervention:

Marc Neeld, marc.neeld@cornwall.gov.uk, Public Health Practitioner (Adv), Wellbeing & Public Health, Cornwall Council.

Second point of contact for this Eoi:

Aisling Algeo, aisling.algeo@cornwall.gov.uk, Public Health Practitioner, Wellbeing & Public Health, Cornwall Council

Supporting Director of Public Health (DPH) or Chief Executive Officer (CEO):

Eunan O'Neill, eunan.oneill@cornwall.gov.uk, Director Public Health (Interim), Wellbeing & Public Health, Cornwall Council.

Expression of Interest

Lead organisation(s):

Cornwall Council

Description of the intervention:

Healthy Cornwall are the in house health improvement service for Wellbeing & Public Health in Cornwall. The all age service delivers a range of services including including stop smoking support, healthy weight, physical activity and a range of training opportunities through a specialised training team.

The service focuses on 7 towns across 3 broad localities in Cornwall where data highlights there are health inequalities in the population. There are teams of Health Improvement Practitioners (HIPs) based in each locality who engage face to face and support local communities. Each team comprises of HIPs who focus on either Maternity and Early Years, Children, Young People and Families and Adults.

A significant proportion of the ring fenced Local Stop Smoking Services Support Grant (LSSSG) has been invested into Public Health to create a new lead role for tobacco control work and into Healthy Cornwall to increase capacity to support people to quit smoking. The Smoke Free Alliance has identified groups who are more likely to smoke, and roles have been developed to support; pregnant smokers who require more complex support and will work closely with the TTD Maternity team; Drug and Alcohol Addiction Treatment Service clients. Community roles have been employed to work in local communities with current teams to target routine and manual workplaces, work with social housing providers and local health partners to target people with Long Term Health Conditions through linking in with local care

NIHR PHIRST local government expression of interest (Eol)

pathways such as targeted lung health checks. Healthy Cornwall are also commissioned to deliver the NHS Treating Tobacco Dependency Programme for people with serious mental illness in the community and potentially the inpatient mental illness programme.

Stop Smoking Services are accessible in a number of ways - One to one support - through advisors, Including Text Support; Telephone Support via weekly phone calls; Digital Support through the use of email or with virtual meetings and group and 121 face to face support.

The aim of the Service is to reduce the number of people who smoke down to under 5% in line with the Governments Smoke Free Generation ambitions. Current prevalence is 12.7%.
We also wish to understand how we could improve the 4 and 12 week quits in the service.

Geographical location(s) of the intervention:

Cornwall wide with a focus on Falmouth, Newquay, Camborne Illogan Pool and Redruth (CIPR), Bodmin, St Austell, Liskeard, Penzance

Current status and timescale(s) of the intervention:

The core Healthy Cornwall offer is ongoing and the LSSSSG work is time limited to March 2029

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

Healthy Cornwall is funded through the core Public Health Grant and the additional roles funded through the Local Stop Smoking Services Support Grant.

Intervention Start Date:

Has been ongoing for a number of years

Intervention End Date:

NA

Impacts:

Reduce the number of people in Cornwall who smoke.
Tackle and reduce the inequalities relating to tobacco use.

Public/community involvement or engagement:

Healthy Cornwall work with the general public on a daily basis

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

Healthy Cornwall

Weblinks:

<https://www.healthycornwall.org.uk/>

Additional information:

Healthy Cornwall data is stored on a single accessible database with a number of years data.

NIHR PHIRST local government expression of interest (Eol)

NIHR PHIRST local government expression of interest (Eol)

Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: NCMP Health Improvement Practitioner / Cornwall Healthy Schools toolkit

Lead Information

Name: Mrs Claire Lowman

Job Position: Advanced Public Health Practitioner

Department: Public Health

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Organisation: Cornwall Council

Contact Information

Lead officer for intervention:

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Advanced Public Health Practitioner, Public Health, Cornwall Council

Second point of contact for this Eol:

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Research Support Officer, HDRC, Cornwall Council

Supporting Director of Public Health (DPH) or Chief Executive Officer (CEO):

Eunan O'Neill

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Director of Public Health, Cornwall Council

Expression of Interest

Lead organisation(s):

Cornwall Council

Description of the intervention:

This evaluation assesses the effectiveness and impact of the newly developed Health Improvement Practitioner (HIP) role within the National Child Measurement Programme (NCMP). The role targets 20 primary schools in Cornwall, selected using three-year Index of Multiple Deprivation (IMD) and NCMP data to identify those with the highest levels of deprivation and obesity in Reception-aged children. Cornwall faces significant challenges with childhood obesity and deprivation. In 2023/24, 25.2% of Reception-aged children were living with overweight or obesity—similar to the national average. Among Year 6 children, those in the most deprived areas had a 23.7% prevalence, compared to 13.2% in the least deprived. These disparities are especially evident in communities where socioeconomic disadvantage limits access to healthy food, safe play spaces, and healthcare. The HIP role aims to address these root causes and prevent long-term health issues.

Using a whole-settings approach, the HIP builds relationships with headteachers, staff, pupils, and families to deliver tailored support. The role promotes healthy behaviours aligned with NICE guidance and the Healthy Schools Toolkit—an online resource developed by Cornwall Council and partners.

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The HIP role aims to:

- Reduce health inequalities by focusing on schools in areas with the highest deprivation and obesity rates.
- Promote healthy behaviours through tailored educational sessions on nutrition, physical activity, and health. These include cooking and activity workshops that build confidence and skills among children, staff, and families.
- Strengthen engagement by offering structured and informal support during school pick-up times and events, fostering trust and participation.
- Encourage behaviour change across school culture, such as healthier snack options, active break times, and embedding health-focused routines.
- Improve access to services by connecting schools with Healthy Cornwall resources, signposting local opportunities, and referring children (with consent) to clinical weight management services when appropriate.
- Embed sustainable behaviours at individual and community levels through a whole-settings approach.

Geographical location(s) of the intervention:

Healthy Schools toolkit is a universal offer to schools across Cornwall but work of the Health Improvement Practitioner targeted to 20 schools using NCMP data and areas of deprivation (IMD)

Current status and timescale(s) of the intervention:

The programme has been in place since the start of 2024 academic year. Funding secured to the end of 2026 academic year. Intention to apply for continuation funding due to success of programme

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

Approved by Public Health Senior Leadership Team to fund from the public health grant. NCMP Health Improvement Practitioner (salary and on-costs) £45534, . No current evaluation costs allocated. All other resource costs are 'in kind' from partnership organisations.

Intervention Start Date:

October 2024

Intervention End Date:

July 2026 but with intention to extend

Impacts:

Population Health

Promotes early healthy habits, reducing future risks of obesity, diabetes, and heart disease. Compassionate approaches support mental wellbeing and positive behaviour change. Identifies children for clinical weight management. Aims to reduce Year 6 overweight/obesity rates below the national average.

Health Inequalities

HIP delivers tailored, equitable support. Builds confidence in underserved students. Increases engagement with Healthy Cornwall, access to free activities, and healthier food choices. Reduces health gaps between most and least deprived families.

Wider Impacts

Improves school food, attendance, and behaviour. Encourages family lifestyle changes. Enhances

NIHR PHIRST local government expression of interest (Eol)

active play and normalises compassionate health conversations across communities.

Public/community involvement or engagement:

The HIP has approached 20 schools; 15 have engaged so far. Of these, 13 are academies/ trusts, with Academy Leads planning wider rollout across more schools. Initial meetings focus on co-design, helping schools assess their health promotion status and shape tailored support. Parent/carer feedback from events and community engagement in a targeted-school town has directly informed the intervention. The Healthy Schools toolkit was co-designed by a steering group including Cornwall Council, schools, and VCSEs. A primary school piloted the toolkit, shaping its development. A bi-monthly newsletter reaches 348 subscribers, collecting topic engagement data and invites feedback to support the toolkit's continuous development. A biennial survey captures pupil voice and informs toolkit updates.

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

Public Health Team
Healthy Cornwall Team
Primary Schools and Multi-Academy Trusts
Healthy Schools Network

Weblinks:

This programme of work links to the Healthy Schools toolkit being offered to all schools across Cornwall
www.healthycornwall.org.uk/organisations/healthy-schools
Cornwall and Isles of Scilly Healthier Weight Strategy (2025-2035)
www.cornwall.gov.uk/media/ljmko5bo/cios-healthier-weight-strategy-2025-35-2800125.pdf
Key components of this strategy are to take a whole systems and compassionate approach for childrens healthy growth and wellbeing

Additional information:

A range of indicators are currently being collected by the Health Improvement Practitioner but we would like to increase the rigor of the evaluation, with ideas for how we might be able to identify comparator schools who have not engaged with the HIP to those who have received full bespoke support. Within the first 6 months the programme has reached 1295 children and 1829 parents/carers.

NIHR PHIRST local government expression of interest (Eol)

Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: A national-local whole systems approach to healthy weight

Lead Information

Name: Ms Sophia Bird

Job Position: Principal Public Health Practitioner

Department: Health and Wellbeing Directorate

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Organisation: Public Health Wales

Contact Information

Lead officer for intervention:

Ms Sophia Bird, sophia.bird@wales.nhs.uk

Principal Public Health Practitioner, Obesity Prevention and Nutrition,

Health Improvement Division, Health and Wellbeing Directorate, Public Health Wales

Lead officer for this Eol:

as above

Second point of contact for this Eol:

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Consultant lead, Obesity Prevention and Nutrition

Health Improvement Division, Health and Wellbeing Directorate, Public Health Wales

Supporting Director of Public Health (DPH) or Chief Executive Officer (CEO):

Professor Jim McManus, jim.mcmanus@wales.nhs.uk

National Director of Health and Wellbeing, Public Health Wales

Expression of Interest

Lead organisation(s):

Public Health Wales

Description of the intervention:

Wales is applying a whole systems approach (WSA) to healthy weight across the nation. Unusually for systems approaches, this intervention is working at national, regional and local level to create system change to support healthier behaviour choices for the population of Wales across the nation.

The approach is embedded in the Welsh Government Healthy Weight: Healthy Wales (2019) 10 year strategy to address obesity. Over 2/3 of the adult population of Wales experience overweight or obesity, and 1 in 4 children in Wales are now starting school with overweight or obesity. The proportion of children with obesity in Wales is higher than that reported for England and Scotland (PHW, 2025).

AIM: Our national to local WSA aims to make it easier for people to access nutritious food, be active, and lead healthier lives, building a foundation for a healthier future across Wales. By recognising the complex factors that influence our health, WSA brings together health organisations, local councils, schools, businesses, and other key stakeholders to create lasting change in our communities.

NIHR PHIRST local government expression of interest (Eol)

A potential strength of this approach is the interface between national and local level system action. Our experience suggests that system challenges that are bigger than one LA can be shared with national colleagues to help develop a national solution through collaboration with Welsh Government colleagues and the directors of Public Health across Wales. In addition, our nationwide approach promotes once-for-Wales solution through the explicit sharing of learning and experiences from one area/ region/ health board to another. This pan-nation approach provides a unique opportunity to evaluate the interface between national and local teams and whether opportunities for system change can be strengthened through a national approach combined with regional and local delivery. Learnings from this approach will be of interest to the other developed nations as well as international colleagues.

Geographical location(s) of the intervention:

Pan-Wales

Current status and timescale(s) of the intervention:

Intervention in process. Part of delivery of 2019-29 Welsh Government Obesity prevention strategy

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

Funded annually via Welsh Government, so, in principle all funding secured. However, funding is per amount agreed in 2019-20. No allowing for pay increases and inflation. Limited funding for evaluation has now been diverted to cover staff pay awards at national and local level, though funding for a regional process evaluation was found 2024-25.

Intervention Start Date:

2021

Intervention End Date:

Mar 2026 but indications from Welsh Government is that funding will continue through 2026

Impacts:

This national-local WSA aims to achieve system change particularly in areas of deprivation where it can be more difficult to make healthier choices. For example, influencing national planning policies and local guidance should help to reduce the increasing number of hot food takeaway outlets (Wales has seen an increase in some areas of 20% over the last 5 years); reducing advertising of HFSS on public sector premises; increasing healthier food options in public sector premises; sharing success in one part of the system by one Local Authority may enable other areas to achieve similar success more quickly; identification of local challenges that connect with national bodies may result in change being delivered across multiple regions.

Public/community involvement or engagement:

Engagement and insight work has been taken forward by each Health Board Public Health team through appreciative enquiry, public meetings, focus groups, surveys, and community events. Part of our 9 steps process to delivering a whole systems approach encompasses connecting and engaging with stakeholders, of which the public are a key part.

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

NIHR PHIRST local government expression of interest (Eol)

Stakeholder involved:

Public Health Wales Health and Wellbeing Directorate,
Welsh Government Healthy and Active Team,
System Leads Principal Public Health Practitioners within each of the 7 Health Boards, their Consultant leads, and their Directors of Public Health

Weblinks:

<https://phw.nhs.wales/topics/whole-systems-approach-to-healthy-weight/>

Additional information:

Though there have been other PHIRST evaluations conducted linked to whole systems approaches, none of them have explored the opportunities and challenges presented by delivering a whole systems approach at a national, regional and local level. The interface between national and local provides opportunities to address system barriers at Government policy, strategy and guidance level as well as promote system change in organisations with a national role, such as SportWales for example.

NIHR PHIRST local government expression of interest (Eol)

Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: Team-Mates: A Community-Based Mental Health Intervention for Men in West Northamptonshire

Lead Information

Name: Mr Chris Serbyn

Job Position: Public Health Practitioner

Department: Public Health

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Organisation: West Northamptonshire Council

Contact Information

Lead officer for intervention:

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Health and Wellbeing Officer

Northampton Town Football Club Community Trust

Lead officer for this Eol:

Mr Christopher Serbyn

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Public Health Practitioner

Public Health - West Northamptonshire Council

Second point of contact for this Eol:

Dr Hong Chen

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Local Authority Research Practitioner

Public Health - West Northamptonshire Council

Supporting Director of Public Health (DPH) or Chief Executive Officer (CEO):

Chloe Gay

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Director of Public Health

Public Health - West Northamptonshire Council

Expression of Interest

Lead organisation(s):

NTFC Community Trust

Saints Foundation

Steelbacks Foundation

West Northamptonshire Council

NSport

Description of the intervention:

NIHR PHIRST local government expression of interest (Eol)

In alignment with the National Prevention Concordat for Better Mental Health, WNC launched a Mental Health and Wellbeing Programme inviting VCSE organisations to propose innovative, collaborative interventions. 4 projects were funded, including Northamptonshire Sport's "Team-Mates" intervention.

Team-Mates responds to the growing need for early mental health interventions targeting men, often underrepresented in mental health services. The programme leverages the trusted platforms of Northampton's 3 sports club's charitable foundations. Northampton Town FC, Northampton Saints RFC, & the Northants Steelbacks combining to reduce stigma, build social connections, & promote wellbeing through sport & community engagement.

Aims:

- Promote positive mental health & wellbeing among men
- Provide accessible, stigma-free early intervention through trusted community settings
- Build sustainable mental health capacity within local sports clubs
- Reduce health inequalities by targeting underserved & at-risk populations

Team-Mates is structured around 3 core components:

The Intervention:

- Weekly walking sport sessions (football, rugby, cricket) to encourage physical activity & social interaction
- Monthly mental health education & support sessions ("Team Talk") to raise awareness & build resilience
- Pre-match "chats" to connect isolated individuals & foster peer support.

Awareness Raising:

- Amplifying national mental health campaigns through club platforms
- Promoting the mental health benefits of physical activity via matchday programmes, social media, & community outreach.

Training:

- Mental Health First Aid, suicide prevention, & safeguarding training for staff & volunteers. Development of mental health champions within clubs to sustain impact.

Evaluation need:

Team-Mates is a scalable, community intervention that aligns with WNC's strategic visions. It combines physical activity, peer support, education, access to services & building community spirits to create a holistic approach to men's mental health. An evaluation will inform future system-wide initiatives & used to support further applications for funding.

We want to understand:

- What are the outcomes beyond improvement in physical activity & mental health?
- How have different sports clubs adapted the intervention to their local context & the needs of different groups of men?
- What are the unintended consequences or ripple effects?
- How has partnership working contributed to its success?

Geographical location(s) of the intervention:

West Northamptonshire

Current status and timescale(s) of the intervention:

NIHR PHIRST local government expression of interest (Eol)

The intervention is active & full mobilisation will be complete by 30th June 2025. The programme runs until 31.03.26 & an evaluation will support further funding of this type between the 3 clubs.

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

Team-Mates is funded by West Northamptonshire Council (£26,200 in Year 1; £63,105 in Year 2) through its Mental Health and Wellbeing Programme. Additional support include £7,176 from Sport England and in-kind contributions from three professional sports clubs.

Intervention Start Date:

December 2024

Intervention End Date:

The project is due to conclude on the 31st March 2026.

Impacts:

Team-Mates aims to improve men's mental health by providing support in community settings to address issues early on & prevent crisis. Through physical activity, peer support & education, & community capacity building, this unique partnership intervention improves men's physical, mental and social wellbeing, their health literacy & ability & willingness to access wellbeing services. It helps enhance capacity of the clubs & foster stronger community cohesion & resilience.

Men often face significant barriers to seeking mental health help through societal norms, cultural expectations, & systemic inequalities. By targeting men at risk of poor mental health, combating stigma associated with mental health, & using inclusive, community-based delivery, the programme addresses health inequalities.

Public/community involvement or engagement:

Whilst there has been no formal public involvement in the design of Team-Mates intervention, the intervention's themes build on existing community-facing programmes & insights from the clubs' extensive previous engagement work, such as Tackling Loneliness Project, Every Player Counts, & walking sports initiatives to name a few.

The council uses these trusted sports clubs as delivery partners because they understand their local culture & communities, & are trusted community spaces. As such, the intervention design & delivery are both community-informed.

Our future work will prioritise structured community involvement to co-design, refine, & evaluate the intervention, ensuring the inclusion of under-served groups, so the intervention continues to meet local needs & reduce inequalities.

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

Northampton Town Football Club Community Foundation - Delivery Partner

Northampton Saints Foundation - Delivery Partner

The Northamptonshire Steelbacks Foundation - Delivery Partner

NIHR PHIRST local government expression of interest (Eol)

West Northamptonshire Council (Public Health) - Commission the intervention
Northamptonshire Sport - This is the active partnership that has received the funding and working with the 3 clubs to deliver the intervention.

Weblinks:

<https://www.northamptonshiresport.org/teammates/>

<https://www.northamptonshiresport.org/wp-content/uploads/2025/04/TEAM-MATES-PROJECT-EVALUATION-REPORT-April-25-1.pdf>

<https://www.ntfcccommunity.co.uk/>

<https://www.northamptonsaintsfoundation.org/>

<https://nccc.co.uk/foundation/>

Additional information:

To our knowledge, funding a partnership between three professional sports clubs in this way is unique to West Northamptonshire. We are particularly interested to understand if using the unique appeal and support for the clubs enhances impactful community engagement.

NIHR PHIRST local government expression of interest (Eol)

Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: No 5 recovery hub, a day centre for street drinkers or people vulnerably housed with alcohol and/or drug issues; No 5 includes a 'wet-room' for safe alcohol consumption.

Lead Information

Name: Ms Mary Hall

Job Position: Consultant in Public Health

Department: Public Health

Email/Phone: mary.hall@leicester.gov.uk 0116 454 2051

Organisation: Leicester City Council

Contact Information

Lead officer for intervention:

Mary Hall

Consultant in Public Health

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Second point of contact for this Eol:

Sam Carruthers

Lead Commissioner, Substance Use

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Supporting Director of Public Health (DPH) or Chief Executive Officer (CEO):

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Director of Public Health

Public Health, Leicester City Council

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Expression of Interest

Lead organisation(s):

Leicester City Council; Inclusion Healthcare

Description of the intervention:

No 5 operates as a day centre for people using drugs and/or alcohol including street and/or dependent drinkers and provides England's (as far as we are aware) only 'wet-room' element, a managed alcohol consumption programme. Different support pathways are provided to service users from case management to informal drop ins, working with partners across the local treatment and recovery system to support individuals in treatment and those not yet ready to access treatment. No 5 has been operating in its current format with its current provider, Inclusion Healthcare, since April 2018 but has operated in a similar guise with various other providers as 'The Anchor Centre' for over 30 years.

NIHR PHIRST local government expression of interest (Eol)

The service aims to:

- a) Reduce alcohol and drug related harm in Leicester residents
- b) Provide a recovery oriented hub for people who use substances and/or live street lifestyles to engage with services.

It's objectives are to:

- a) Provide a safe, accessible and therapeutic environment for individuals to engage with the service, specialist staff onsite and relevant partner agencies.
- b) Provide a menu of support including individual case-managed support, informal drop-in and group based interventions.
- c) Identify people not currently engaged in substance use treatment and actively refer them, where appropriate, to partner treatment agencies.
- d) Delivery and manage a 'wet room' including the safe monitoring of alcohol consumed on site, supporting street drinkers to have a safe space to drink.
- e) Develop, maintain and strengthen recovery communities for individuals accessing the service.
- f) Support relevant stakeholders to delivery a range of services within No 5 including specialist substance use treatment services, homeless services and harm reduction services.

Geographical location(s) of the intervention:

Hill street, Leicester city centre.

Current status and timescale(s) of the intervention:

The service, in its current guise, has been running since 2018 and has just been re-procured for a further 4 (3+1) years.

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

The service is funded by Leicester city council, public health as part of the ring fenced public health grant. Funding for some additional activities has been provided by OHID drug and alcohol grant money as a supplement to the core service function. Core funding is @£190k per year. No current resources have been allocated to an evaluation however it may be possible to allocate some funding from next year's OHID grant money.

Intervention Start Date:

The service has been running since April 2018.

Intervention End Date:

Current funding is for three years from April 2025 with an option to extend for a further one year.

Impacts:

Primary impacts of No 5 focus on harm reduction: supervised consumption with an aim of gradual reduction; non-street drinking with risk of violence or arrest; access to healthcare services including fibroscanning for liver damage, hepatitis C testing; access to treatment, recovery and peer mentors. Alcohol consumption does not affect everyone equally and Leicester has one of the highest rates of self-reported abstinence in the country yet one of the highest rates of alcohol related mortality. No 5 aims to impact this alcohol harm paradox in some of our most vulnerable citizens. Wider impacts include those on the city centre: reductions in street drinking and violence, making the city feel a 'safe and attractive' place to visit.

Public/community involvement or engagement:

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No 5 works closely with other treatment providers and recovery organisations in the city. This includes staff and volunteers in recovery who help to shape the service and activities within it.

Wider community engagement occurs with those businesses surrounding No 5 particularly around increasing understanding of its purpose and role.

Before the service moved to its current location there was widespread stakeholder, community and political engagement enabling it to exist in its current form.

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

Inclusion Healthcare Social Enterprise CIC who are commissioned by public health to run No 5.

Turning Point: the main drug and alcohol treatment provider in Leicester City who work closely with No 5 and who have a regular presence in the service.

Dear Albert (The Stairway Project) a lived experience recovery organisation in the city who work closely with No 5 and who have a regular presence in the service.

Sarah Harrison, City Centre Director

Wider potential stakeholders include Leicestershire Police, surrounding businesses, Hepatitis C trust, Unity House (a housing project for people in abstinence).

Weblinks:

<https://www.inclusionpractice.co.uk/health-information/service-directory/no-5-hill-street/>

<https://www.inclusionpractice.co.uk/>

<https://www.dearalbert.co.uk/>

Additional information:

NIHR PHIRST local government expression of interest (Eol)

Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: Calling it out- Loudmouth

Lead Information

Name: Ms Isabel Monger

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Organisation: Wandsworth Borough Council

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Better Service Partnership

Richmond and Wandsworth Councils

Lead officer for this Eol:

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Director of Public Health

Public Health Division

Better Service Partnership

Richmond and Wandsworth Councils

Expression of Interest

Lead organisation(s):

NIHR PHIRST local government expression of interest (Eol)

Wandsworth borough council

Description of the intervention:

Background: 'Calling it Out' is a school-based theatre in education programme using performance pieces and interactive facilitator lead workshops. The programme, run by Loudmouth Education and Training, is delivered to pupils aged 13+ and increases young people's awareness of harmful sexual behaviours, supporting them to safely call out and report incidences of sexual harassment and assault. The programme allows children and young people to explore contentious and difficult issues in formats that support early help, safeguarding and relationship education. Theatre is used to 'tell a difficult story' and is proceeded by reflective question and answer sessions and smaller workshops to dissect issues and practice healthy relationship skills in an interactive and safe environment.

Rationale: The programme covers issues such as harmful sexual behaviours, sexual harassment, sexual assault, pornography, consent and misogyny. These areas are closely linked to gender-based violence. Violence against women, in particular sexual violence and intimate partner violence, is considered by the WHO to be a major public health problem and negatively impacts all aspects of women's health: physical, mental, sexual and reproductive. The Richmond and Wandsworth 2024 Sexual and Reproductive Health needs assessments make recommendations relating to gender based violence prevention, support and care and the importance of comprehensive education and information in improving sexual health outcomes. 'Calling it out' specifically addresses various key actions in our Sexual and Reproductive health strategies 2025-2030 that are informed by the needs assessments. The programme links to the national PSHE guidance that states key aspects of the law relating to sex should be taught in schools including consent, rape, sexual assault and harassment.

Key Aims:

The primary aim of the programme is to raise young people's awareness of harmful sexual behaviours and to:

- Explore where harmful sexual behaviours come from and promote positive behaviours.
- Increase awareness of the impact of everyday incidences of sexual harassment and assault.
- Promote how everyone has a role in safely challenging and reporting these behaviours.

Additional public health aims:

The programme contributes to the wider health improvement and health inequalities goals of public health through prioritising schools where there are:

- Incidences of sexual harassment/abuse
- Higher Pupil Premium and/or Free School Meals

Geographical location(s) of the intervention:

Secondary schools across the London Boroughs of Richmond and Wandsworth.

Current status and timescale(s) of the intervention:

Loudmouth Programmes are currently running in Richmond and Wandsworth and will continue to be run for the next 4 years across 13 secondary schools.

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

Loudmouth were commissioned by the Councils as a proof of concept programme to deliver Calling it Out as one of three theatre in education programmes offered to schools by public health. Total funding for the programme is £224,000 over the next 4 years. Schools contribute to 20% of funding costs and the public health grant covers 80%. The programme is offered to 13 secondary schools.

The programme is currently evaluated through pre and post surveys on a subset of students who have taken part in the programme. There are no other additional resources allocated toward evaluation.

NIHR PHIRST local government expression of interest (Eol)

Intervention Start Date:

September 2022

Intervention End Date:

August 2029

Impacts:

Population health and health inequalities impacts:

- Decreased risk of exposure to abuse and harassment with decrease in long-term associated ill health risk of sexual abuse and harassment.

- PHOF indicator B12c: Violent crime - sexual offences per 1,000 population

- Sexual violence victims are predominantly women and young girls (ONS) long term impacts are reduced health inequalities between genders that sexual abuse and harassment perpetuate.

Specific impacts:

Evaluative pre/post programme questionnaires have found:

- Increased awareness and knowledge around assault, abuse and harassment.

- Increased skills in spotting the signs of assault and harassment.

- Increased confidence in reporting assault and harassment.

- Increased access to support and services.

- Increased safeguarding disclosures.

Public/community involvement or engagement:

Research and engagement with young people and professionals is conducted regularly to ensure programmes remain current, relevant and factually accurate. Engagement uses focus groups, 1:1 interviews and online surveys. Scripts are anonymously based on real life scenarios and dialogue that children, young people and professionals report.

Loudmouth draws on audience reactions and evaluation of the dramas and workshops to develop and fine tune content and delivery styles. This includes presented scenarios, accompanying music, characters and their experiences, and script dialogue (appropriateness/relevance/credibility of language characters use). Actor/facilitators feedback to Loudmouth operational and creative team following each session. This informs content, programme and script adaptation.

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

Loudmouth Education and Training, Public Health Richmond and Wandsworth

Weblinks:

<https://loudmouth.co.uk/programmes/programme/calling-it-out>

https://www.richmond.gov.uk/media/q01odny/richmond_sexual_and_reproductive_health_needs_assessment_2024.1

https://www.richmond.gov.uk/media/icvbauph/richmond_sexual_and_reproductive_health_strategy_2025_2030.pdf

https://www.wandsworth.gov.uk/media/msfpfu51/wandsworth_sexual_and_reproductive_health_needs_assessment.p

https://www.wandsworth.gov.uk/media/upbhkuvu/wandsworth_sexual_and_reproductive_health_strategy_2025_2030

Additional information:

This Eol seeks to critically evaluate 'Calling it Out' which has not yet undergone academic evaluation.

Other programmes offered, have been academically evaluated as valuable, preventative educational tools that do benefit students. Evaluating Calling it Out will add to our knowledge base.

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The lead organisation is listed as Richmond and Wandsworth borough councils. The councils operate under a shared staffing agreement with public health serving both boroughs.

NIHR PHIRST local government expression of interest (Eol)

Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: Suffolk Healthy Homes Partnership: improving housing conditions to improve health and wellbeing

Lead Information

Name: Miss Emma Brinkley

Job Position: Project Manager - Natural Environment and Climate Change

Department: Public Health and Communities

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Organisation: Suffolk County Council

Contact Information

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Lead officer for this Eol:

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Second point of contact for this Eol:

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Supporting Director of Public Health (DPH) or Chief Executive Officer (CEO):

Stuart Keeble, Stuart.Keeble@suffolk.gov.uk, Director of Public Health, Public Health & Communities, Suffolk County Council.

Expression of Interest

Lead organisation(s):

East Suffolk Council (ESC)

Description of the intervention:

Suffolk has an ageing population of around 760,000 (2021 census) living in circa 352,000 homes, in conditions worse than the England average when assessing the presence of significant hazards to health (category 1) in the private sector. 14% of Suffolk residents are in fuel poverty.

Since 1999, Suffolk Councils have worked together to deliver Healthy Homes programmes aimed at alleviating fuel poverty, improving health, and addressing poor housing conditions, successfully securing £millions in external funding to support energy efficiency and retrofit initiatives.

Our latest Suffolk Healthy Homes programme (led by ESC) focuses on Core20PLUS5 areas—targeting individuals in the poorest health living in the worst housing—with support beyond the physical measures installed. Residents face multiple challenges that only become clear during home visits; our “make every contact count” approach signposts to additional support, e.g. benefit checks, utility advice, befriending

NIHR PHIRST local government expression of interest (Eoi)

services, decluttering help, and adaptations for disabled residents. Programmes are being further bolstered through training, guidance and a Suffolk Healthy Homes portal, to raise health professionals' awareness of the health impacts of housing/indoor air quality and increase referrals.

Current evaluation includes home energy performance using the Standard Assessment Procedure score (Energy Performance Certificate, EPC), and general customer feedback including self-reported changes in mental and physical health. However, we lack robust evaluation of any resulting health improvement or reduction in health service demand, which would strengthen the case for further investment. Retrospective analysis of health record data pre-/post-installation offers one evaluation option.

However, a 2025 Warm Homes-Local Grant (WH:LG) award of £4.6 million to support energy performance upgrades and low carbon heating in the worst performing, low-income households (80-100 per year over 3 years) provides an opportunity to holistically evaluate the health impacts of accessing the Suffolk Healthy Homes programme from the point of referral and, where residents have health conditions exacerbated by poor housing, evaluate health before and after housing improvements (which may include insulation, upgraded heating, better ventilation, and reduced energy bills). We will identify the target cohort through established work with key GP practices and a data sharing protocol with the Norfolk and Waveney ICB.

Geographical location(s) of the intervention:

Principally Suffolk wards within the 20% most deprived in England (2019 IMD) in addition to some rural properties due to the nature of rural deprivation) which isn't reflected in the IMD data).

Current status and timescale(s) of the intervention:

June to August 2025: WH:LG customer eligibility being determined

August 2025 to March 2026: First tranche of retrofit assessment of works and installs

March 2028: last tranche of installs completed

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

Installations prior to June 2025 were funded by the 24/25 Home Upgrade Grant and ECO 4. Those from June 2025 will be funded by the WH:LG. A Public Health Grant funds a Health and Housing Manager to build partnerships and expand the Suffolk Healthy Homes programme. Existing resources will continue to collect energy performance data and basic customer feedback. PHIRST could enable a retrospective evaluation of health record data (pre-/post-installation), or real-time holistic evaluation of the HWB impacts of receiving home improvements and/or accessing the wider Healthy Homes programme support.

Intervention Start Date:

WH:LG commences June 2025

Intervention End Date:

First tranche of retrofit installations will be completed by March 2026. The wider WH:LG programme continues until March 2028.

Impacts:

Housing affects health. Poor quality housing costs the NHS £1.4billion each year through treating preventable illness like asthma, respiratory disease, and fall-related injuries, and driving many health inequalities. We aim to improve peoples' homes for better health and wellbeing: alongside improved

NIHR PHIRST local government expression of interest (Eoi)

home energy efficiency scores, a better quality home will improve resident health, and ultimately reduce healthcare demands and costs to the NHS through reduced medication use, reduced demand for health services (e.g. lower attendance rates to GPs/A&E), and improved self-reported mental health scores. By targeting the 20% most deprived homes, we hope to improve health outcomes in those most likely to be living in poor quality housing at most vulnerable to the risk of health inequalities.

Public/community involvement or engagement:

Public and community engagement has included roadshows, social media promotion, signposting to the Warm Homes Suffolk website, letter drops, direct / targeted marketing.

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

All Suffolk district and borough councils, Suffolk and North East Essex ICB, Norfolk and Waveney ICB, GPs, East Coast Community Health, University of Suffolk, Voluntary sector organisations including Citizens Advice

Weblinks:

<https://www.gov.uk/government/publications/warm-homes-local-grant>

<https://www.warmhomessuffolk.org/>

<https://awards.lgcplus.com/2025/en/page/public-health-2025>

<https://www.healthysuffolk.org.uk/healthy-you/suffolk-healthy-homes/healthy-homes>

Additional information:

East Suffolk Council is the winner of the 2025 East of England Council of the Year in the National Energy Efficiency Awards and is a finalist in the Local Government Chronicle Awards 2025 in the Public Health category.

There has also not been a PHIRST evaluation in the East of England to date.

NIHR PHIRST local government expression of interest (Eol)

Call Title: 25/107 PHIRST LA Eol March 2025

Name of intervention to be evaluated: Bexley School Superzone (Slade Green) Evaluation

Lead Information

Name: Mrs Zoe Wright

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Organisation: London Borough of Bexley

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Director of Public Health

Public Health

London Borough of Bexley

Expression of Interest

Lead organisation(s):

London Borough of Bexley

Description of the intervention:

School Superzones establish a 400m radius around schools to create a healthier environment for

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children, young people, and their families. These zones encourage healthier food options in local shops, active travel such as walking and cycling, cleaner air, and safer streets by reducing traffic. They also restrict advertising of unhealthy products like junk food, alcohol, and gambling.

Bexley currently has two School Superzone areas (Thamesmead and Slade Green); this project focusses on the Slade Green SSZ area where a number of new initiatives have been implemented. The evaluation will help to consider the effectiveness of these initiatives prior to further expansion of the School Superzone programme in Bexley.

As part of the Slade Green School Superzone, we have launched a range of projects aimed at fostering healthier, safer, and more connected communities:

- **Slade Green Active Travel Programme:** this encourages students to adopt active travel options—such as walking, cycling, or using public transport—to and from school. This initiative not only promotes physical activity but also helps reduce vehicle emissions in school areas, creating a cleaner and safer environment.
- **Road Safety Project:** this aims to address key safety concerns through influencing local authority transport planning. Our goal is to ensure that active travel routes to and from schools are safe and accessible, encouraging more families to choose sustainable modes of transport.
- **Friends of Peareswood Community House Project:** this seeks to transform a local primary school space into a valuable community resource. This project will provide food aid and a communal area where residents can connect and support one another.
- **St Paul's Meadow Project:** this has provided Forest School training to the school community, helping to enrich outdoor learning experiences for children and fostering a deeper connection with nature. This will be achieved by actively engaging with the community, fostering meaningful connections among both children and adult residents. Through interactive events, collaborative projects, and open dialogue, we aim to create an inclusive environment where everyone feels valued and heard.

Geographical location(s) of the intervention:

Slade Green and Northend Ward - 400m radius around each school to make one School Superzone. Schools include: St Pauls Primary School, Peareswood Primary School, Haberdashers Aske's Primary School

Current status and timescale(s) of the intervention:

Started April 2024 - March 2026

Availability of funding:

Has all required funding to deliver the intervention been secured? - Yes

Funding:

The Slade Green School Superzone was initially funded for two years by the Greater London Authority (GLA) (up to 31 March 2024). Following completion of the initial project, an evaluation of outcomes was undertaken by the GLA. The additional interventions for 2024/25 onwards have been fully funded by the Local Authority's Public Health grant. Slade Green Travel Project £12,000; Friends of Peareswood Community House Project £30,000; Forest School Training £1,000; Road Safety Project – Funded by LBB Highways

Resources have not been allocated to an evaluation of the projects listed above.

Intervention Start Date:

April 2024

Intervention End Date:

March 2026

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Impacts:

The reduction of health inequalities through community engagement, creating solutions in partnership. E.g, the active travel project will encourage children to walk, cycle, or scoot to school, lowering the carbon emissions around schools, improving the air quality and boosting the physical activity. The road safety project, will create safer road crossings near schools, to support children and adults to actively travel. The Community House Project will create a community space for residents to create a happier, safer, and healthier environment, where families thrive and children and young people feel supported in their education and every day lives. This will also link with other initiatives within the local community such as food growing spaces within a local open space.

Public/community involvement or engagement:

There has been significant engagement with local schools, school staff, children and parents as part of this project. Local ward councillors have been engaged in the development and implementation of projects and have retained an ongoing interest in progress. Local community groups have also been engaged to ensure alignment with wider community projects. The following engagement has taken place or is planned:

- Voice of the child focus groups with year 5 & 6 children (scheduled for July 2025)
- Parent/carer survey (scheduled for July 2025)
- Stakeholder engagement meetings (ongoing)
- Articles in The Bexley Magazine
- Termly School Superzones newsletters to schools
- Individual meetings with schools
- Local community groups – regular updates on progress

Stakeholder consent:

Do all stakeholders, including the lead organisation, consent to being involved in an evaluation, if it goes ahead? - Yes

Stakeholder involved:

School Superzone schools:

- St Paul's Primary School
- Peareswood Primary School
- Haberdashers Askes Primary School

Other stakeholders:

- Friends of Peareswood
- Community House Project manager
- Be Cycle – local organisation commissioned to lead the Active Travel project
- Local ward councillors
- London Borough of Bexley Public Health team
- London Borough of Bexley Highways team
- Bexley Food Alliance
- Bexley Voluntary Sector Council
- North Bexley Local Care Network

Weblinks:

https://www.london.gov.uk/sites/default/files/superzones-_final.pdf

<https://www.london.gov.uk/programmes-strategies/health-and-wellbeing/school-superzones>

<https://www.bexley.gov.uk/services/health-and-social-care/bexleys-public-health/school-superzones>

Additional information:

An evaluation of the School Superzones programme was undertaken by the Greater London Authority

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(GLA) following completion of the GLA-funded programme in March 2024. A number of new projects have been initiated in Slade Green since this evaluation was completed. London Borough of Bexley is currently considering further expansion of the School Superzones programme and would welcome evaluation of the additional initiatives to inform our future planning.